7 Two Feet Under and a Cardboard Coffin

The Social Production of Indifference to Child Death

A child died today in the favela. He was two months old. If he had lived he would have gone hungry anyway.
Carolina Maria de Jesus (1962:108)

The opposite of love is not hate, but indifference.
Elie Wiesel (1990:174)

Forebodings

"Why do the church bells ring so often?" I asked Nailza de Arruda soon after I had moved into a corner of her tiny mud-walled hut near the top of the Alto do Cruzeiro. It was the dry and blazingly hot summer of 1964, the months following the military coup, and save for the rusty, clanging bells of Nossa Senhora das Dores Church, an eerie quiet had settled over the town. Beneath the quiet, however, were chaos and panic.

"It's nothing," replied Nailza, "just another little angel gone to heaven." Nailza had sent more than her share of little angels to heaven, and sometimes at night I could hear her engaged in a muffled, yet passionate, discourse with one of them: two-year-old Joana. Joana's photograph, taken as she lay eyes opened and propped up in her tiny cardboard coffin, hung on a wall next to the photo of Nailza and Zé Antônio taken on the day the couple had eloped a few years before. Zé Antônio, uncomfortable in his one good, starched, white shirt, looked into the camera every bit as startled as the uncanny wide-eyed toddler in her white dress.

Nailza could barely remember the names of the other infants and babies who came and went in close succession. Some had died unnamed and had been hastily baptized in their coffins. Few lived more than a month or two. Only Joana, properly baptized in church at the close of her first year and placed under the protection of a powerful saint, Joan of Arc, had been expected to live. And Nailza had dangerously allowed herself to love the little girl. In addressing the dead child, Nailza's voice would range from tearful imploring to angry recrimination: "Why did you leave me? Was

your patron saint so greedy that she could not allow me one child on this earth?" Zé Antônio advised me to ignore Nailza's odd behavior, which he understood as a kind of madness that, like the birth and death of children, came and went.

It was not long after that Nailza was again noticeably pregnant, and the nightly prayers to Joana ceased, momentarily replaced by the furtive noises of stolen marital intimacies. By day, Nailza's appetite and her normally high spirits returned, much to my relief. The peacefulness was, however, soon rent by the premature birth of a stillborn son. I helped Nailza dig a shallow grave in our *quintal*, the trash-littered excuse for a backyard where pigs and stray goats foraged and where we hoped to dig a pit latrine before the start of the winter rains. No bells would ring for this tiny fellow, nor would there be any procession of the angels accompanying his body to the graveyard. Stillbirths remained (in those days prior to hospital delivery for Alto women) outside the net of public and medical surveillance. And when curious neighbors commented the next day on Nailza's flat stomach, she tossed off their questions with a flippant "Yes, free and unburdened, thanks be to God!" Or with a sharp laugh, she would deny having been pregnant at all. Even living with Nailza in our close quarters, I had a hard time knowing what she was experiencing in the weeks and months that followed, except that Joana's photo disappeared from the wall, and her name was never again mentioned as long as I lived in that house. The stillborn son returned Nailza to her senses and to an acceptance of the reality in which she lived. Neighbors would say approvingly that Nailza had learned to *se conformar* to the unalterable conditions of her existence. But at what price I wondered, at what physical, psychic, and social cost to Nailza and other women like her and at what risk to their seemingly unbroken succession of "replacement" babies and subsequent angel-children?

At the time I first posed them, these questions held only immediate and pragmatic value for me. As a community health worker specializing in maternal and child health, I wanted to know just what was necessary to propose and introduce "appropriate" kinds of intervention. Should sick-to-death infants be treated at home or in the clinic? Could one teach young mothers how to administer intramuscular injections of water-salt-sugar solution under the infant's clavicle as a dangerous but desperate "last ditch effort" at saving severely dehydrated babies? Would collectivizing child care through a cooperatively run day care center allow for the simultaneous rescue of Alto children and their mothers? It was the 1960s, and I struggled to maintain a positive view of shantytown life, which (prior to my training in cultural anthropology) meant the attribution of all forms of human misery and suffering to purely exogenous causes—poverty, racism, class exploitation,

political and economic imperialism, and so on. And like so many of my North American compatriots, with our terrible fear of human difference, I projected onto my Alto friends and neighbors a comforting illusion of familiarity and sameness. We were all very much alike, very much the same (I liked to think) beneath the superficial veneer of cultural differences.

Both views eventually proved themselves inadequate, and I clung to them in the face of a good deal of cognitive incongruity. But I managed to bury the conflict in a mad whirl of activities, in attending to the multitude of small, day-to-day crises surrounding the work of the shantytown association and the building (later the running) of the cooperative day nursery, the creche. There was no time for abstract, philosophical reflection and musings on the human condition, on the nature of human nature, or on the varieties of response to the stresses of chronic hunger, deprivation, sickness, and premature death. Then, too, there was always the specter of depression and despair hovering anxiously at my shoulders, spurring me on to further activity, often at the expense of understanding.

Buried, perhaps, but never completely banished, for when I returned to Brazil for the first time in 1982, I came back to understand events that had puzzled me then and that continue even today, following four more field trips, to elude easy or definitive interpretation. My primary research questions were overdetermined at the outset. They were the ones that first crystallized during a veritable die out of Alto babies during a severe drought in 1965. The food and water shortages and the political and economic chaos occasioned by the military coup of the spring of 1964 were reflected in the spidery, difficult to read, handwritten entries on births and deaths in the yellowed pages of the huge ledger books kept at the *cartório civil*. Almost five hundred babies died in the *município* that year, and three hundred of these came from the shantytown population of less than five thousand individuals. But it wasn't the deaths that surprised me. It would take no Rudolf Virchow to crack some mysterious epidemiological puzzle. There were reasons enough for the deaths in the miserable conditions of shantytown life. Rather, what puzzled me was the seeming "indifference" of Alto women to the deaths of their babies and their willingness to attribute to their own offspring an "aversion" to life that made their deaths seem wholly natural, indeed all but expected. I shall try to explain what I mean.

Within the first month of my arrival in Bom Jesus, a young mother came to me with a very sick and wasted baby. Seeing that the child's condition was precarious, I rushed with him to the local hospital, where he died soon after, the desperate efforts of myself and two clinic attendants notwithstanding. I was devastated and frightened. I had come from a society in which (at least to my limited knowledge and experience) babies didn't die, or if they did,

where it was a great human tragedy for all concerned. How could I break the news to the child's mother? Would she hold me responsible for the death? Would I be forced to leave my post of duty so soon after my arrival? Selfish concerns, mind you. Meanwhile, I had to trek all the way through town and climb the Alto with the tiny, yet strangely heavy, dead weight in my arms. It was almost more than I could bear, and I wept bitter and angry tears all along the way. To my great wonder and perplexity, however, the young woman took the news and the bundle from my arms placidly, almost casually and indifferently. Noting my red eyes and tear-stained face, the woman turned to comment to a neighbor woman standing by, "*Hein, hein, coitada! Engraçada, não é*; Tsk! Tsk! Poor thing! Funny, isn't she?" What was funny or amusing seemed to be my inappropriate display of grief and my concern over a matter of so little consequence. No one, least of all the mother, had expected the little tyke to live in any case.

Later that afternoon I returned to the home to attend the brief and understated wake. The small group of women and children who had gathered was mainly concerned with hearing me speak my then-fractured Portuguese, and gales of laughter followed my awkward attempts at expressing myself. Many had never heard a foreigner speak their language before, and they judged my accent completely unintelligible. The baby in its sky blue cardboard coffin was balanced precariously on a simple, straight-backed kitchen chair. No one had bothered to clear off the table, which was cluttered with dishes and knicknacks. A small knot of barefoot children played dominoes on the floor, and when a fight broke out among the players, the coffin was very nearly knocked over during the fray. No one corrected the children as they bumped into the chair, rocking its silent cradle. After an hour or so had passed, an older woman (she may have been the child's grandmother) called on some of the children to form a procession of angels to carry the baby to the cemetery. A couple of older girls were chosen as pallbearers. One refused, saying that she was "too old." Another complained that she was "always" the one chosen, and she pointed to another little girl, who ran off to hide, giggling. As the procession was about to leave, I asked whether a prayer would be recited. "It's only a baby," one of the women scolded, while the children bounded off down the hill, with the unfastened lid of the coffin flapping madly up and down. "Slow down!" a neighbor called to the children from her open door. "It's not a circus, you know."

Like the women who were listening to me and not understanding a word, I stood there dumbly, hearing and seeing but failing to grasp the meaning of the social drama being played out before me for the first of many times that year. We were strangers to each other and mutually unintelligible. I was faced, although I did not have the words for it then, with the opacity of

culture. My only point of reference was the casual indifference and alienation implied in Mersault's opening lines in Albert Camus's *L'étranger*, which I had read for a French class earlier that year. The words returned to me with a new force and power: *Aujourd'hui, maman est morte. Ou peut-être c'est hier, je ne sais pas.* "Mother died today, or possibly yesterday. I don't know [or care, it seemed]" (1955:21).

What had made death so small, of such little account on the Alto do Cruzeiro? Had the exposure to altogether too much sickness, hunger, and loss turned these women's hearts to stone? Because the indifference appeared in women who were ordinarily vibrant, emotionally charged, and sometimes quite sentimental when it came to remembrances of things and relationships past, it required some explanation.

In this first of three chapters on child death and mother love, I begin with a discussion of the "routinization" of child death in the creation of an "average expectable environment" of child death, meaning a set of conditions that places the infant at great jeopardy of sickness and death, accompanied by the normalization of this state of affairs in both public and private life.[1] In the otherwise highly bureaucratic world of Bom Jesus, child death has yet to seize the imagination of political leaders, administrative and civil servants, physicians, and priests or religious officials as an urgent and pressing social problem about which "something must be done." Rather, there is a failure to see or to recognize as problematic what is considered to be the norm (as well as normal, expectable) for poor and marginal families. Michel Foucault (1975, 1980) has written of the hostile gaze, the punitive net of surveillance cast by the state and its disciplinary and biomedical technicians over the sick and deviant majority. Here, I am writing about an averted gaze, the turning away of the state and its agents in their failure to see, to acknowledge what should be right before their eyes.

In subsequent chapters I move from the public to the private realm, from *rua* to *casa*, to explore the conditions that normalize infant deaths within the family and that place all infants at high risk. The apparent indifference of Alto mothers toward the deaths of some of their infants is but a pale reflection of the "official" indifference of church and state to the plight of poor mothers and children. Although the routinization of child death in the miserable context of shantytown life is easy to understand, its routinization in the formal institutions of public life in Bom Jesus is difficult to accept uncritically. Here, the "social production of indifference" (see Herzfeld 1991) takes on a more malevolent, even sinister, cast.

I begin by sketching some historical and demographic features of child mortality worldwide and for Brazil as a whole, and I relate these broader patterns to the specific ethnographic instance: Bom Jesus da Mata during the twenty-five-year period 1964–1989. Drawing on municipal statistics and hospital records and on the reproductive histories of a large sample of Alto women, I show the interlocking threats of micro- and macroparasitism in maintaining a high level of what demographers refer to as "reproductive wastage" in poor women.

The Discovery of Child Mortality

Throughout much of human history, as in a good deal of the developing world today, women have had to give birth and nurture infants under environmental and social conditions inimical to child survival as well as hostile to their own well-being. Consequently, a high rate of infant and child mortality has been a fairly standard feature of human reproduction (see M. Cohen 1989:130–131). The dialectic between birth and death, survival and loss, remains a powerful one in the lives of most people living on the peripheries of the modern industrialized world, either as marginalized rural workers or as urban shantytown dwellers. In these contexts disease epidemics, food shortages, contaminated water, and inadequate medical care interact with patterns of high fertility and sometimes prejudicial forms of infant care to consign millions of children to an early grave.

In the world in which most of us live, the dialectic between fertility and mortality has lost its edge and is buried in the back of our consciousness. For most Europeans and North Americans each birth signifies new life, not the threat of premature death. But it was not so long ago in our own "Western" world that reproduction was as unpredictable and death as "random" and chaotic as they are in Northeast Brazil today. In many rural areas and in the squalid tenements of the urban slums of nineteenth-century England and America, there was hardly a family that had not experienced firsthand the death of an infant or a small child. Public health and sanitation, child and adult labor laws, and social welfare legislation had yet to vanquish the great uncertainties of human existence. A walking tour through any New England churchyard, with its symmetrical rows of infant tombstones, gives silent testimony to the fickleness of life to those who lived and died on the threshold of the great epidemiologic and demographic transitions.[2]

The mundaneness and ubiquitousness of child death—a fairly permanent feature of the history of childhood until quite recently—contributed to an array of private and public responses and to individual and collective defenses. One of the most common of these was the failure to recognize childhood mortality as a significant personal or social problem and to "naturalize" and normalize an infant death rate that, in various times and places, had sometimes reached 40 percent of all live births. Consequently, childhood mortality rates were difficult to discern up through the mid- to late-

nineteenth century for Europe and North America. Often they were not tabulated apart from adult deaths. Child mortality remained hidden insofar as it had yet to achieve the status of a social or a medical problem in which the state had a clear vested interest.

One obstacle to the "discovery" of child mortality was the tendency of medical professionals and public officials alike to treat infant death as normal and routine, part of the "natural order" of things. This was reflected in the way the British Registrar General classified the causes of deaths in infants and small children. Many of these deaths were tabulated under the heading "diseases of growth, nutrition, and decay," which included deaths from "old age," "debility," and "natural causes" along with deaths from "wasting" (usually childhood malnutrition) and "prematurity" (Armstrong 1986:221; Wright 1988:306). It was only in 1907 that birth registration became compulsory in England (Armstrong 1983:15); about the same time the country launched its first national campaign against infant and childhood mortality. With the passage of the Registration Act of 1834, every death had to be publicly recorded as to its cause in the form of a medical disease so as to replace the so-called natural deaths of the old and of babies. An infant mortality rate was created in 1875, thus suggesting for the first time a social awareness of these premature deaths and a recognition of the infant as a discrete entity (Armstrong 1986:212).

Pediatrics, meanwhile, was a relative latecomer to medical specialization: the British Pediatric Association was founded in 1928, and in the United States pediatric medicine developed only when women began entering the medical profession and the medical academy at the turn of the century. Relatedly, childhood malnutrition and diarrhea, two of the greatest killers of young children, were not identified as pediatric diseases until the twentieth century. In the first instance, "misrecognition"—the failure to understand the social relations of infant wasting as hunger (rather than as a congenital or constitutional defect in the child)—was responsible for the omission. In the second instance, professional self-interest led British doctors to suppress diarrhea as a diagnostic category; because diarrhea was deemed a "trivial" illness by the general population, physicians feared that parents might attribute incompetence to those doctors involved in the treatment of fatal pediatric cases.

It is ironic that the discovery of childhood malnutrition (first identified as a pediatric disease in 1933) and the more specific diagnoses of marasmus, kwashiorkor, and PEM (protein-energy malnutrition) had to await Western medical doctors encountering these disorders in the tropics and adopting a traditional Ghanaian term as their clinical diagnostic category. Kwashiorkor is an Ewe term for an infantile wasting disease that is caused by an insufficient intake of protein, producing apathy, edema, and partial loss of pigmentation. It is generally associated with stunted growth, delayed development, and chronic diarrhea, and it is recognized as a serious, potentially fatal, childhood disorder. In the native etiological system kwashiorkor is recognized as a socially produced disease of childhood: it is said to be the disease of the "deposed" child, who, following the birth of a younger sibling, is weaned from the breast to a low-protein maize diet. Protein-calorie malnutrition in children (of which there was an epidemic in nineteenth-century England [see, for example, Engels (1845) 1958:160–190]) only entered medical nosology when British doctors working in the colonies discovered it as a "tropical" disease.[3]

In short, the social construction, or (as Armstrong preferred) the social "invention," of child mortality and, later, child survival as important social and medical problems about which something must be done is of fairly recent vintage. In many parts of Europe and in North America up through the mid-nineteenth century, high infant mortality was not regarded as intolerable or unacceptable but rather as a fairly predictable and expected occurrence. For one thing, child mortality had always had a clear class reference that allowed it to remain hidden: the children of the rural and urban poor and working classes were mostly afflicted. As Foucault wrote: "It was of little importance whether *these* people lived or died, since their reproduction was [thought of] as something that took care of itself in any case" (1980:126). It would take class and political conflict and economic emergencies (such as the new labor requirements of a more advanced stage of urban industrialization) to provoke state interest in the regulation of fertility and the control of population, including a concern with child mortality and child survival, which, in recent years, have achieved the status of a "master" social and political problem in the developing world.[4] And so gradually the "naturalness" of infant death was called into question, and the infant was no longer regarded as "one of death's natural habitats, but rather as a terrain in which death was an obscene intrusion" (Wright 1988:306).

But in many Third World countries mired in a relatively crude form of dependent capitalism and still characterized by a high mortality and a high, "untamed" fertility, the naturalness of infant and child mortality has yet to be questioned, and parents may understand a baby's life as a provisional and undependable thing—a candle whose flame is as likely to flicker and go out as to burn brightly and continuously. There, child death may be viewed less as a tragedy than as a predictable and relatively minor misfortune, one to be accepted with equanimity and resignation as an unalterable fact of human existence.

Following from a high expectancy of loss, reproductive and child-tending

habits may be based on a thinking in sets, on a presumption of the interchangeability and replaceability of offspring (see Imhof 1985). Reckonings of the social, moral, and economic value of the individual child may be measured against those of older children, adults, or the family unit as a whole. These moral evaluations are in turn influenced by such "external" contingencies as population pressures, subsistence strategies, household composition, cultural ideas concerning the nature of infancy and childhood and definitions of personhood, and religious beliefs about the soul and its immortality, all of which are explored in the following pages.

I wish to make it clear from the outset that the apparent indifference of Alto women toward the lives and deaths of some of their infants is continuous with, and a pale reflection of, the official bureaucratic indifference of local agents of church and state to the problem of child mortality in Northeast Brazil today. The social production of official indifference is similar to what Bourdieu (1977:172–183) meant by *meconnaissance*, or "misrecognition," the largely unconscious rejection of the "unthinkable" and "unnameable," especially those social relations that must be concealed and misread so as to guarantee complicity in the collective bad faith alluded to earlier. What cannot be recognized in this instance are the social determinants of the overproduction of *Nordestino* angel-babies.

Child Death in the Northeast: The Colonial Context

In his *Guia Médico das Maẽs de Família*, published in Rio de Janeiro in 1843, J. B. M. Imbert cautioned the mistresses of the large estates of the Northeast to exercise caution in selecting *babás* and wet-nurses for their newborns from among their female slaves living in the traditional *senzalas*. While suggesting that black slave women were more physically suited to the task of lactation than were white and free women (1843:89), Imbert also noted the unhygienic infant care practices of slave women that contributed to a high infant mortality in the slave quarters:

> The Negro women ordinarily cut the cord very far from the navel and are more and more inclined to follow the pernicious custom of putting pepper on it and covering it with castor oil or some other irritant. Having done this the perverse creatures squeeze the child's belly to the point of suffocating it. This cuts the life-thread of many, many young ones and contributes to the development in the navel of that inflammation which goes by the name of the seven day sickness. (Cited in Freyre 1986a:381)

In addition, Imbert noted that slave women tended to ignore the "weak digestive organs of the newborn" and to feed infants a gruel made up of the same kinds of "coarse food such as the mothers themselves are accustomed to eating." He warned the plantation mistresses to be on their guard against black nursemaids bringing their dangerous practices inside the big houses. But he also advocated that the mistresses try to educate their slaves so as to curb these practices in the slave quarters as well, for "the Negro women that give birth are augmenting their master's capital" (cited in Freyre 1986a:382).

The concern with the slave master's primary means of production promoted some interest in the question of black infant mortality in the colonial period and may have contributed to the negligible differences in the birthrates and infant death rates of the slave and free populations as recorded for a number of *Nordestino* plantations and *fazendas* in Brazil for the late eighteenth and early nineteenth centuries. Freyre cited, for example, a census taken in the sugar plantation community of São Antônio, Pernambuco, in 1827 that showed comparable adult and child death rates for free whites, free mulattoes, and enslaved blacks. Nevertheless, even if the differences between the free and slave child mortality were negligible (and Freyre's selective use of questionable historical sources must certainly be questioned), child mortality was still extremely high in both *casa grande* and *senzala*. The premature death of infants and their mothers for many successive generations in the colonial and postcolonial periods "reduced by something approaching fifty percent the production of human beings in the Big House and in the slave hut" (Freyre 1986a:385). This was reflected, for example, in the writings of Maria Graham, the British diarist who traveled through Brazil in the early nineteenth century. She recorded in her published *Journal of a Voyage to Brazil* that, according to the mistress of an *engenho* in Mata-Paciência, less than half the Negroes born on that estate lived to be ten years old.

The high infant death rate of the slaves was affected by the "economic circumstances of their domestic life and labor" (Freyre 1986a:377). Many slave women who worked in the fields were forced to carry their infants and small babies strapped to their backs in cloth slings. One chronicler cited by Freyre, F. A. Brandão, Jr., told of an estate master in Maranhão who obliged his female slaves to deposit their infants in the field shack on plantation lands for the duration of their mothers' workday. The infants were suspended in hammocks or left alone on floor mats, while more mobile toddlers were sometimes buried up to their chest in holes dug in the ground so as to curtail their movements over long stretches of time.

By contrast, the high mortality in the big house was due largely to the failure of the Portuguese colonists to adapt their traditional childrearing practices to a new environment. The European planters brought with them

to the New World the practices of swaddling and head bundling that contributed to the death of infants exposed to the often sweltering heat of the tropics. In his *Memórias Históricas de Pernambuco*, published in 1844, Fernandes Gama noted that although the original Portuguese planter class suffered an extremely high infant mortality ("two-thirds of them died shortly after birth"), successive generations of plantation mistresses eventually learned to adopt more practical approaches to infant care, and they did not suffer as much "reproductive waste" as their mothers and grandmothers had.

But even though Brazilian infant and child mortality decreased in Brazil beginning in the second half of the sixteenth century, it remained high. Child death was first elevated to a national concern in Brazil during the late nineteenth century, when it became one of the problems addressed by the hygienists of the Second Empire. At a meeting of the Brazilian Academy of Medicine held in Rio de Janeiro in 1846, the subject of child mortality was raised for general discussion and debate. The physicians in session directed their attention to two main issues: child mortality and pediatric illnesses. Their explanations of child mortality ranged from climatic considerations (dampness, humidity, tropical airs, radical changes in temperature), to inappropriate pediatric feeding practices, to the exposure of vulnerable infants to tropical diseases. Virtually all the conference participants implicated the institution of Negro slave wet-nursing in the high infant mortality. The black nurse was accused of transmitting syphilis and other venereal and skin diseases to the suckling infant via contact with her skin and contaminated milk. These physicians also attacked as unhygienic the common slave practice of hand-feeding infants with premasticated adult foods, and they criticized the *babás'* customs concerning the care of the umbilical cord, infant head shaping, underdressing of babies, and the use of traditional herbal remedies for common childhood sicknesses.

The first Brazilian medical text to deal exclusively with the problem of child mortality was José Maria Teixeira, *Causas da Mortalidade das Crianças no Rio de Janeiro*, published in 1887. It remains a unique statement of late-nineteenth-century social medicine. In contrast to the prevailing medical opinions of his day, Teixeira laid the blame for high childhood mortality on the political economy of slavery itself and all the perversions that it reproduced in both the big house and *senzala*. Teixeira cited the great disparity in the ages of married couples that was customary among the plantation elite, the poor education and poor maternal preparation of both white and slave mothers, and the frequency of unwanted, illicit births produced in master-slave sexual relations. As an aside he noted that child death was not treated as a great misfortune in the big house because plantation women married young and there would always be another newborn to replace a dead sibling.

Child Death Today: The Modernization of Child Mortality

> Finally, we come to the question of what advantage can be taken [on behalf of child survival] of the prevailing winds of free enterprise now blowing across both developing and industrialized worlds. . . . The decade of the 1980s will perhaps . . . be remembered as the decade in which economic theories began to change with remarkable suddenness. Enterprise systems and market mechanisms are now being more fervently embraced by almost every nation, including many with long marriages to other ideologies.
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> James P. Grant (1990:65)

Approximately one million children younger than five die each year in Brazil, or about forty children every hour. An estimated 25 percent of all infant deaths in Latin America occur in Brazil, and of these more than 50 percent take place in the Nordeste, which has an estimated infant mortality rate of 116/1,000 live births, one of the highest in the hemisphere and comparable to the poorest parts of Africa (IBGE 1986:38). Official statistics are, however, at best an approximation of an underreported phenomenon.[5] The inefficiency of basic public health and medical services in the Northeast is such that an estimated two-thirds of those infants who die do so without a medical diagnosis.

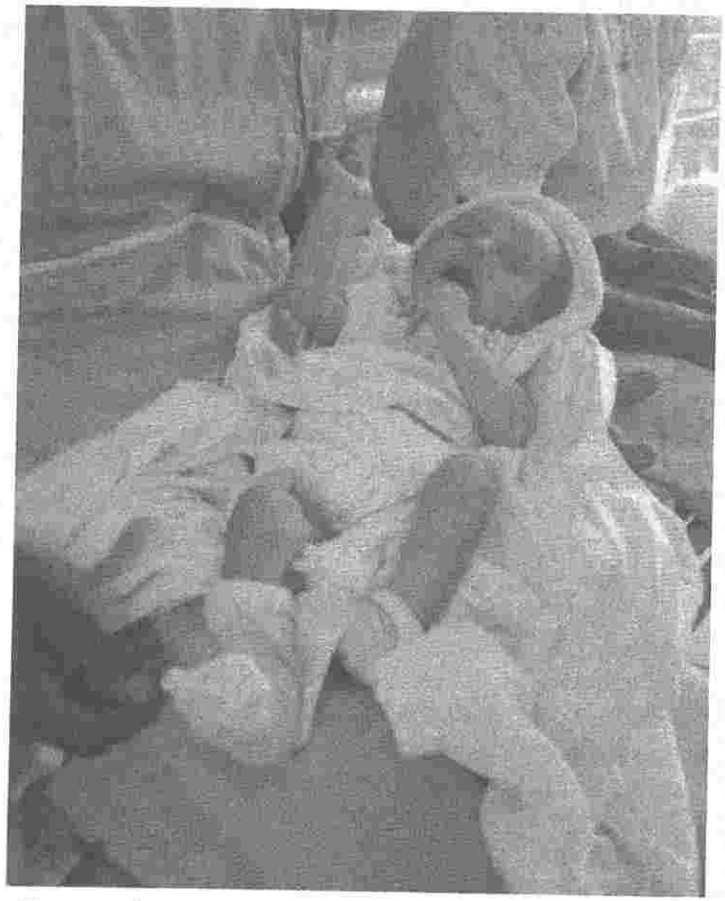
Hence, counting dead infants in Northeast Brazil is every bit as daunting as U.S. census workers' attempts to count the homeless in American cities. Much of the problem is tucked away from public scrutiny. Nonetheless, this is a startling reality coming from the world's eighth largest economy, the largest in the Southern Hemisphere. Insofar as infant and child mortality rates may be taken as a particularly sensitive barometer of how well or how badly a nation (region, community, or household) is faring at a given time, what accounts for the persistent and perversely high child mortality in Brazil as a whole and for the Northeast in particular?

One tendency has been to explain the high rates of child death and disease in the Third World as the almost inevitable consequences of impersonal ecological, climatic, or demographic conditions, such as the perennial droughts of the Brazilian Nordeste or the "tropical" climate of the coastal regions, with its plagues of microparasites and "exotic" infectious diseases. Alternatively, explanation is sought in the "rampant," uncontrolled fertility of the poor, which leads to food shortages and gross pediatric malnutrition. The followers of classic demographic transition theory identify economic and social "underdevelopment" as the key culprit, and they predict—based

on analogy with the recent history of Europe—that with advances in industrialization and with the penetration of modern, capitalist modes and relations of production into the "backward" hinterlands of Brazil, both the high infant mortality and the high "replacement" fertility will decline. That such has not been the case for Brazil, which over the past four decades of phenomenal economic growth and despite a marked decline in fertility has maintained a high infant mortality (indeed, in some cases, the economy and infant mortality have expanded in tandem), requires some explanation. What each of the ecological, population studies, and demographic transition theory approaches has obscured is the role of pernicious class relations in the social production of child morbidity and mortality. In other words, the foregoing equations have failed to note the macroparasitism of uncontained "market forces" that has fed and preyed on the bodies of the young, the vulnerable, and the powerless. Economic expansion and "development" have been waged in Brazil largely at the expense of the general social welfare and sometimes at the price of child survival. Hence, I refer to the "modernization" of child death with the same irony with which feminists describe the "feminization" of poverty in the United States in the late twentieth century.

And so despite the modernization of the Brazilian economy and its transformation in the second half of the twentieth century into a world-class superpower, glossed in the late 1970s as the Brazilian economic miracle (see Singer 1972), the rates of infant and child mortality did not show everywhere the expected decline. In fact, the mortality and fertility rates increased in some years for the most populous centers of the country. It is true that one observes for Brazil *as a whole* an initial and precipitous decline of 50 percent in child mortality between 1940 and 1970 and a decline in the average number of births from 5.8 in 1970 to about 3.3 in 1985 (see Berguó 1986). But when one begins to break down these aggregate data, certain patterns emerge that tell a different story, one that I refer to as the modernization of child mortality. By this, I mean, on the one hand, the "taming," "standardizing," and containing of child death to one level of society—the very poorest, especially those millions of rural exiles now living in the teeming shantytowns and *mocambos* that encircle São Paulo, Brasília, Belo Horizonte, Rio de Janeiro, Recife, and Forteleza. On the other hand, I mean the replacement of the "old" childhood killers, especially tetanus and other infectious childhood diseases now controlled by immunization, by the "new" killers, especially infant malnutrition and diarrhea-caused dehydration, both related to bottle-feeding. Under the new childhood mortality pattern, death comes to children at an even earlier age.

The "old" pattern of child mortality that characterized Brazil from the



The new infant mortality: death by dehydration.

colonial period up through the mid-twentieth century was one of exceedingly high mortality affecting all social classes and races in Brazilian society. Epidemics of infectious diseases—measles, smallpox, malaria, pneumonia, and various tropical fevers—carried away the infants and small children of rich, poor, black, white, and mulatto alike. In addition, the poorer populations suffered the double jeopardy of malnutrition and gastrointestinal problems related to their impoverished and unsanitary living conditions. The rapid industrialization, urbanization, and modernization of Brazilian society and social and public institutions during the twentieth century have brought about the expected demographic and epidemiological transitions for the middle and upper classes and initially for the urban working classes as well. In addition to greatly improved standards of living, these social classes benefited during the decade of the 1970s from expanded and improved

medical care, immunization, birth control and day care facilities, and possibly the world's largest national breast-feeding promotion campaign, initiated in March 1981.

These same social and biomedical forces had little or no impact on the morbidity and mortality of Brazil's poorest popular classes, while the economic "revolution" affected them adversely. This explains certain discrepancies and contradictions in the nation's changing epidemiological profile. Never again would black and white, rich and poor, master and worker, patron and client, bureaucrat, civil servant, and peasant alike share the same risk of child morbidity and mortality that Freyre described for the colonial and postcolonial period in Brazil. Child death had been vanquished in the *casas grandes, praças,* and main *ruas* of town and country. But it has not disappeared; child death has retreated to the back streets, muddy roads, and squalid hillsides of Brazil, where it has become the "privilege" of the *favela* and the shantytown alone. And there it will remain, requiring more than UNICEF's program of universal immunization, breast-feeding promotion, and oral rehydration (ORT) to dislodge it. Here, the social and economic roots of child death run very deep and very wide, indeed. They drink from the polluted groundwater produced by the unleashing of the same market forces cautiously welcomed in the 1990 UNICEF report *The State of the World's Children* (Grant 1990).

Consequently, one finds in Brazil today two contradictory epidemiological profiles, one for the rich and middle classes and the other for the poor. It is as if history had bifurcated, producing the expected demographic transition for part of the country, leaving the rest to die the way they always had: of sickness, hunger, and gross neglect.

And so beginning in 1966, two years after the Brazilian military coup, infant and child mortality began to rise in some regions. W. Leser (1972), for example, examined the evolution of child mortality in São Paulo between 1909 and 1970, noting the general tendency toward decline until 1961. After that date, but especially in the early 1970s, the rates rose again. Later, Roberto Macedo (1988) examined the effects of specific indices of the growing economic crisis in Brazil on the health, welfare, and mortality of children in the state of São Paulo throughout the 1980s. Rising rates of urban unemployment, worsening income distribution, and decline or stagnation in the production of basic food staples over the period 1975–1985 were highly correlated with the rise in infant mortality, a marked increase in the percentage of low-birth-weight infants in São Paulo's two major public hospitals, and a sharp rise in the number of abandoned street children, especially during 1982–1984.

In 1970 the Pan American Health Organization (PAHO) directed a large, comparative study of child mortality patterns (see Puffer & Serrano 1973) in fifteen rural and urban sites in the Americas (Argentina, Brazil, Colombia, Bolivia, Canada, Chile, El Salvador, Jamaica, Mexico, and the United States). The researchers identified Recife, the capital of Pernambuco and a mere ninety kilometers from Bom Jesus da Mata, as having the highest rates of infant mortality, of child deaths from malnutrition, and of infant deaths related to low birth weight of all urban centers sampled. Recife also had the poorest and most crowded living conditions. Even so, the PAHO investigators did not include the many congested shantytowns and peripheral neighborhoods that encircle the capital. We can well imagine that the *actual* infant mortality rate of Recife was considerably higher than the reported 91.2/1,000 live births. The woman identified by the PAHO investigators as the most likely to have lost an infant during the study years—an illiterate rural migrant who was marginally employed—could be any Alto woman older than thirty-five.

A team of physicians and epidemiologists at the University of Salvador, Bahia (see Paim, Netto Dias, & Duarte de Araújo 1980), closely monitored child mortality in that city (the fifth most populous in Brazil) and found that following a reduction by 53 percent in infant mortality between 1940 and 1966, the rate of decline was reduced dramatically until 1968, when mortality began to rise, reaching its highest point in 1971. Using multiple regression analysis, the team was able to identify the factors that most influenced child morbidity and mortality in Salvador for the period 1962–1973. Health care and access to pediatric assistance were less significant than real wages, water supply, and educational level.

Similarly, Charles Wood (1977), an American researcher working for the Ford Foundation in Brazil, where he was also affiliated with a large planning and development agency in the city of Belo Horizonte, investigated the relationship between the conservative economic policies of the military government and the increase in infant mortality in two of Brazil's most important and populous urban centers, São Paulo and Belo Horizonte. He argued that in the years following the coup of 1964, the military junta supervised a transfer of the national income from the poorest 40 percent to the richest 10 percent of the population, thereby precipitating a decline in the standard of living for the largest number of people; the epidemiological consequences were disastrous. During the 1960s infant mortality in São Paulo rose more than 40 percent; in Belo Horizonte the rate of child death increased almost 70 percent between 1960 and 1973. There was, Wood discovered, a direct and negative correlation between real wages and infant mortality: as family income and purchasing power of workers declined in São Paulo and Belo Horizonte during the 1970s, the rates of infant mortality

correspondingly increased (Wood 1977:58–59). Meanwhile, of course, development and business were "booming" in Brazil.

Because the rise in infant mortality occurred in Brazil's capital cities, such as Recife and Salvador in the Nordeste, and in regional metropolises, such as São Paulo and Belo Horizonte, in the industrialized south, the richest part of the country, economic development is not of itself predictive of a child survival revolution. In fact, it is the young who have suffered the brunt of the cost of economic expansion. Part of the cost of servicing Brazil's massive foreign debt has been severe cuts in public spending, including child health, welfare, and education programs.

Here, then, is the net result of Latin American nations' headlong rush at midcentury into the World Bank version of economic development: in the 1980s the economies of Latin America suffered their most severe setback in fifty years, which erased the economic boom of the 1970s (Albanez, Bastelo, Cornia, & Jespersen 1989:1). Of the ten Latin American countries for which there were reliable data, only one nation, Cuba, showed an unambiguous and steady decline in rates of infant mortality throughout the 1980s. Cuba, which already had the lowest mortality rates for the Southern Hemisphere, continued to advance its own child survival campaign successfully, so that from 1980 to 1986 the rates decreased from 16.6/1,000 to 13.6/1,000, just when the rates for infant mortality were rising or were stagnant for other Latin American nations because of inflation, foreign debt, and declining real wages for the working classes (Albanez et al. 1989:36). In this instance, Cuba's isolation from Western market forces and from the World Bank's economic development policies for Latin America served it well with respect to insulation from the long-term side effects of those policies on the health and well-being of the population.

The case of Brazil is particularly illustrative. The share of the government's total expenditures in interest payments in servicing the foreign debt rose from 7.2 percent in 1980 to 29.4 percent in 1985, while total expenditures in health decreased from 7.8 percent to 6.4 percent in 1985, just when infant mortality rates were beginning to show a significant rise (see Chahard & Cevini 1988). Real minimum wages fell 33 percent between 1981 and 1988, while food prices rose, so that the number of hours of labor at the minimum wage necessary to feed a family increased by more than a half in some years. On the Alto do Cruzeiro, where as much as 75 percent of family income is spent on basic food purchases, the net result was hunger. This was most marked during 1982–1984 when cornmeal began to replace beans and rice as a staple food, much to the detriment of child health and nutrition in particular.

The irregular and incomplete epidemiological transition that I am describing for Brazil as the modernization of child mortality is perhaps not so very different from what occurred in Europe at the beginning of the nineteenth century after the introduction of smallpox vaccination. There, the causes of death from smallpox among infants and small children were "taken over" by increased deaths caused by gastrointestinal illnesses (Imhof 1985:4). As René Dubos (1960) warned against the conceits of biomedical science, the advance against disease is never won, as one new pattern of morbidity and mortality comes to supplant the old, and the utopia of health or survival for all remains as elusive as a mirage in the desert.

In the interior of Northeast Brazil today, many poor infants and babies still manage (despite vigorous immunization campaigns) to escape vaccination until they reach school age, but they are generally protected against the old scourges of childhood—measles, smallpox, diphtheria, whooping cough, and so on—by the immunization of the majority, which reduces the risk for all. But there is no immunization against malnutrition and chronic diarrhea. Oral rehydration therapy can "save" a poor infant on the brink of death a half dozen times or more until, finally, the little one simply refuses and dies of hunger several weeks or months later. Jairnilson Paim, an epidemiologist at the Federal University of Salvador, and his colleagues estimated that only 2 percent of the Northeast Brazilian infant mortality rate can be reduced through immunization (see Paim, Dias, and De Araújo 1980:332).

The question to be asked is whether it is really an advantage to be rescued from one sickness only to be killed later of another more prolonged and painful one? We can ask ourselves a related question in light of the epidemiological transition that has occurred in our own society over the past hundred years as death from infectious disease—from polio and tuberculosis and from pneumonia-influenza-bronchitis—has been largely displaced by death at a later age from heart disease and cancer. Death from infectious disease is often premature but quick. Pneumonia was long and affectionately called the "old man's friend" in the United States because it brought a relatively quick death to the sufferer. Death from the "new" chronic diseases of old age are prolonged, painful, and both socially and psychologically troublesome, as one must observe the slow, yet irreversible, loss of the body's capacities and functions as well as the diminution of one's social roles, social network, and economic resources. These are large moral and ethical concerns, questions that are being debated by bioethicists (see Callahan 1990) and discussed in medical schools throughout the country. Adults can express their wishes through living wills, in malpractice law suits, and in the columns of the nation's newspapers. They can, at least hypothetically, have some say about how they *wish* to die.

The infants and babies of Northeast Brazil have no say. They cannot

protest the international child survival programs and campaigns that can cruelly (although unintentionally) prolong their suffering and death. I try in the following pages to articulate the suffering of those—both women and children—who are too young to speak or whose words and opinions have been ignored or discredited and yet whose bodies are on the line. But how can one initiate a dialogue (especially in the case of infants) with the speechless? Only, perhaps, in the wordless space and primitive solidarity of "being there" and "bearing witness" to the suffering of the silent or silenced others—in this instance, mothers and babies. With all its pretensions and all of its difficulties, I know of no other way to proceed.

The Overproduction of Angels: Keeping Track, Losing Count

> The child shall be registered immediately after birth and shall have the right from birth to a name.
>
> Article 7, United Nations Convention on the Rights of the Child (1989)

Throughout Northeast Brazil, whenever one asks a poor woman how many children she has in her family, she invariably replies with the formula, "*X* children, *y* living." Sometimes she may say, "*Y* living, *z* angels." Women themselves, unlike the local and state bureaucracies, keep close track of their reproductive issue, counting the living along with the dead, stillborn, and miscarried. Each little angel is proudly tabulated, a flower in the mother's crown of thorns, each the sign of special graces and indulgences accumulating in the afterlife. There are a great many angels to keep track of. It is just as well that so many women are doing the counting.

When I first began in 1982 to try documenting the extent of infant and child mortality in the *município*, I was stymied by the difficulty in finding reliable local statistics. I was referred by various public officials of Bom Jesus to the office of the local IBGE, the national central statistics bureau. This was a small rented room across the hall from a local dentist's office in "downtown" Bom Jesus and was closed each time I went there. Finally, one afternoon I encountered a civil servant sleeping in a chair in the otherwise empty office. There was not so much as a typewriter or a file cabinet. "No," I was told, "there are no statistics kept here—no numbers at all." Everything, I was told, was tabulated and sent off to the central office in Recife.

The application of some local political pressure, however, yielded summaries of vital statistics for the community for selected years in the 1970s. In 1977, 761 live births (599 in hospital, 162 at home) and 311 deaths of infants were recorded, yielding an IMR (infant mortality rate) of 409/1,000. In 1978, 896 live births (719 in hospital, 177 at home) and 320 infant deaths yielded an infant mortality rate of 357/1,000. If these statistics were reliable, they indicated that between 36 percent and 41 percent of all infants in the *município* were dying in the first twelve months of their lives, a state of affairs that was immediately and roundly denied by the mayor as an absurdity. "My *município* is growing, not declining," he insisted, and Seu Félix sent me to the local hospital to corroborate the IBGE statistics with the records on births and deaths kept there.

The Barbosa Hospital and Maternity Center is one of three hospitals serving the entire region of the *zona da mata—norte* of Pernambuco and Paraíba. Although privately owned by the Barbosa family, the hospital primarily serves the needs of the rural popular classes, many of whom receive medical services without charge. Hence, the hospital attracts a large clientele that extends far beyond the limits of the municipality, and its statistics reflected a regional, not a municipal, pattern. Nonetheless, the head nurse gave me access to her records. For 1981 a total of 3,213 deliveries were recorded, of which 807 were of indigent, or nonpaying, patients. The remaining deliveries were covered by the national health care security system or by the rural workers health fund. There were 98 (3.1 percent) stillbirths and 38 (1.2 percent) perinatal deaths in the maternity wing for that year. When I returned in 1987 the figures for the previous year were 2,730 deliveries, of which 68 (2.5 percent) were stillborn and 27 (1 percent) died within forty-eight hours postpartum. Official death certificates were issued in the name of attending physicians, but these were generally filled out by a nurse or hospital functionary. And the causes of death, when given at all, were perfunctory: "prematurity" and "heart and respiratory failure" were the most common diagnoses. One hospital physician had a disproportionately high number of stillbirths and perinatal deaths in his practice at the maternity wing. But no one seemed to be keeping track too closely or carefully.

After I had begun, through various and sometimes creative means, to assess the extent of child mortality in Bom Jesus, I made a visit to the first and newly appointed secretary of health for Bom Jesus. Responding to inquiries about the greatest health risks to the population of the *município*, the debonair and energetic Dr. Ricardo offered without a moment's hesitation, "Stress." And he began to outline his proposals for a stress-reduction education program that would target the substantial business and professional class of the community. Heart problems and cancer were, the secretary of health continued, the two greatest causes of death in the bustling little metropolis. When confronted with the data painstakingly culled from the civil registry office in Bom Jesus indicating that almost a half of all deaths in the *município* each year were of children under the age of five and that

diarrhea, not heart disease, and hunger, not stress, were the main pathogens, Dr. Ricardo sighed and raised his eyes to the heavens: "Oh, child mortality! If we were to talk child mortality . . . an absurdity, surely. And unknowable as well."

"What do you mean?"

"When I took over this office last August, the municipal administration had no figures on child mortality, none whatsoever. I had to send for them from the state, and they were unusable: an infant mortality of 120 percent!"

"How can that be?"

"And why not? It's quite straightforward. The official figures said that of every 100 infants born in Bom Jesus, 120 of them died before they reached the age of one year! What a disaster! No wonder we are so underdeveloped in Brazil—more of us die than are even born!"

"Surely there are other ways of counting the dead," I suggested. "For example, how many charity baby coffins does the mayor's office distribute each month?"

"Oh, there's no limit there, no limit at all. We give the people as many as they want. In fact, the more they want, the better! It's one of the things we take care of very efficiently and well."

The doctor was pulling my leg, of course, but his remarks captured both the social embarrassment and the bureaucratic indifference toward child mortality as a premodern plague in a self-consciously modernizing interior town.

Later that day I stopped in again to visit Seu Moacir, the municipal "carpenter," although what he "carpenters" for the city are poor people's coffins, mostly baby coffins. Nonetheless, Moacir strongly objected to being called the municipal coffin maker or having his crowded annex to the back of the municipal chambers referred to as a coffin workshop or a *casa funerária.* And so the discreet sign over his door read, "Municipal Woodworks." But even here there was some deception at play, for the media in which Moacir worked were as much cardboard and papier-mâché as plywood and pine. His "product," he told me, cost the city between two and eight dollars apiece, depending on size.

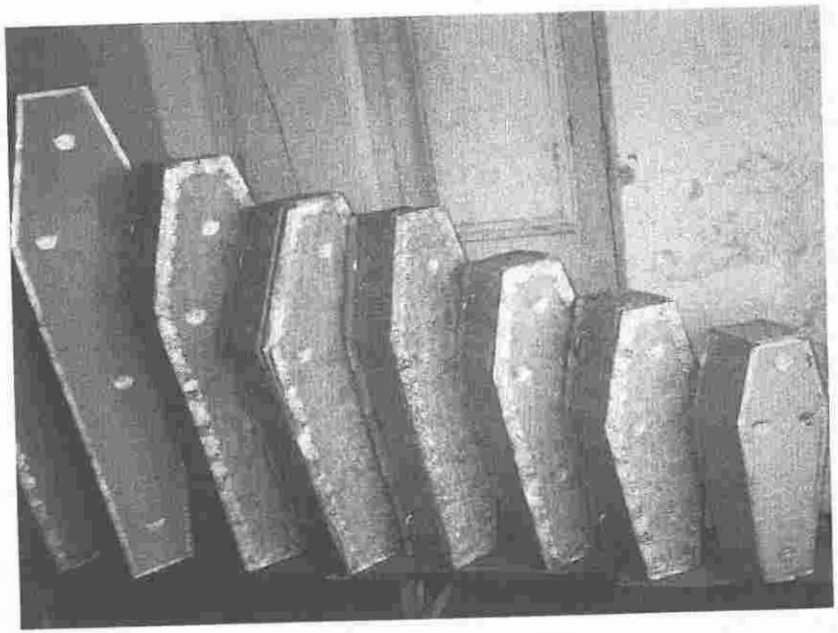
Yes, he was quite busy, Moacir said, but he could answer a few questions. He has been the municipal carpenter since 1965, when Seu Félix decided that every citizen had the right to a decent burial. There were more than twice as many baby coffins requested as adult ones. February and March were the "busiest" months for his work. Why? Perhaps it was, he hazarded a guess, because people liked to marry in June after the *festas juninas* were over and boys and girls on the Alto had begun to "pair up." Moacir was a man of few words, and his own curiosity in the matters I was raising was limited. But



Seu Moacir, municipal "carpenter."

the craftsman in him readily agreed to pose for pictures, and he held up both an adult coffin and a baby one, pointing out that the style was similar for both—a cardboard top and a plywood bottom.

All adult coffins, regardless of sex, were painted a muddy brown ("Earth tone," said Moacir), and all children's coffins, males and females, to the age of seven were painted "sky blue, the favorite color of the Virgin." Moacir noted a detail: there were no fasteners on the children's coffins because parents preferred to put their angels into the ground as unencumbered as



Children's coffins: all in a row.

possible so that the children's spirits were free to escape their premature graves. Moacir found it difficult to estimate how many coffins "left" the workshop each week: "Some days as many as five or six will leave the shop. And then there are days when there are no requests at all." But, he added, "this doesn't affect my productivity. I just keep on working steadily so that coffins are never lacking in the *município*. I don't like to fall behind in my work; even on a holiday a comrade can find me, and I will have a coffin in stock that will serve his needs."

I asked Moacir if he would be willing to go over his requisitions for the previous few weeks, and, somewhat reluctantly, he agreed. We moved over to a cluttered desk with slips of paper in small, untidy piles. "Here," he said picking up one pile, "I'll read them out to you. But I warn you, things are a little chaotic. Here's one: baby, female, three months, June 22, 1987." And he continued, "Newborn, male, June 17, 1987. Female, about six months, June 11, 1987. Male, four months, June 17, 1987."

Then something had him stumped, and he had a hard time reading the slip of paper. As I approached him to look at it myself, he put it down abruptly: "This has nothing to do with anything. It's an order for seventeen sacks of cement! I warned you that everything was all mixed up here."

When I learned that all the requisition orders were referred back to Seu João in the town hall, I approached João himself for access to the records on

all materials furnished by the *prefeitura*. Grumbling, Seu João got down the ledger books, but he warned me not to trust any of them: "If you want numbers," he suggested, "just double everything that's put down here—our inventory is incomplete." In the books that documented in neat columns the "movement" of all supplies in and out of the *prefeitura*, the data on baby coffins were there, interspersed with data on Brillo pads, light bulbs, chlorine bleach, kerosene, toilet paper, cement, alcohol, and soap. In a six-month period in 1988 the *prefeitura* had distributed 131 free infant and child coffins.

When I asked Seu João, who was delighted to get his books back so quickly, why the data on baby coffins were not kept separately, he replied, "Because it wouldn't be of interest to anyone." The deaths of these children, like their brief lives, are invisible and of little or no account.

Finally, I was referred to the *cartório civil* of Bom Jesus, a small, airless, and windowless office privately owned and run by the formidable Dona Leona and her humorless twenty-year-old son. Here, for a small fee, the vital statistics of the community—births, deaths, marriages, and (since 1986) divorces were registered by hand in one of several large ledger books. I was invited to borrow one of the two chairs and a small space at a desk to count the entries for selected years, an occupation that took the greater part of many mornings in Bom Jesus da Mata in 1987 and later again in 1988 and 1989.

Dona Leona has maintained the *cartório* for thirty years, and she worked in "cooperation" with the mayor's office. The town hall furnishes a space in the municipal cemetery and a charity coffin only to those who have registered a death at the *cartório civil*. Consequently, the data on child deaths since 1966 are fairly complete, with the exception of stillbirths and perinatal deaths, many of which are neither registered nor buried in the municipal graveyard. Late abortions and stillbirths, many of them occurring at home on the Alto do Cruzeiro, are buried privately in the *mato* or in the backyard, and there is no question of a medical record or a death certificate. Moreover, until relatively recently, the deaths of unbaptized babies of any age went unregistered. As "pagan" infants they were stigmatized creatures and were buried covertly by their parents at a crossroads in the country, the place where Exu, the Afro-Brazilian deity, and his host of unbaptized spirit infants congregate to serve as messengers for good and ill in the world.

In addition to these, about one-fifth of all Alto births today still take place at home, keeping the half dozen elderly midwives (called *parteiras* or *curiosas*) fairly regularly employed. The *parteiras* who work today in virtual isolation from the medical institutions of Bom Jesus (following years of unsuccessful attempts to regulate them and to incorporate them into the

extension work of the state health post) especially fear running afoul of the "bureaucracy" and the medical profession in Bom Jesus. Consequently, the *parteiras* do not encourage registration of infant births or deaths in which they were involved. Moreover, stillbirths and perinatal deaths are roundly denied by the midwives, who actively compete with each other for an ever-constricting market.

Although, with these exceptions, the data on infant mortality at the *cartório civil* can be taken as fairly complete and reliable, there is no possibility of ascertaining a reliable infant mortality *rate*. Although universal birth registration is mandatory today, there is no way to enforce it at the local level. In practice, most poor families delay registration until the child has to confront the "state" for the first time—usually on registering for primary school. Otherwise, an individual may not be registered until he or she wants to enter the work force, marry, join the military, or receive some medical or social benefit from the state. Moreover, although all registrations of infant deaths (to age one) also require birth certificates, registrations of child deaths older than one year do not require that the births of the children also be registered. Somewhat more reliable, then, is a calculation of the proportion of child to adult deaths for the community as a whole for the years selected for study. I selected the three years advisedly: 1965, the year following the Brazilian military coup and, as I recalled it, the year of the great die out of Alto babies; 1985, twenty years later and following the crash of the great economic miracle; and 1987, the period of democratization and preparation for the transfer of authority from military to civilian rule, also the year of the cruzado novo and significant fiscal reforms aimed at restructuring the economy with respect to inflation and the debt crisis.

Public records, whether official censuses, birth or baptismal certificates, marriage or divorce records, or death or burial certificates, are obviously not "neutral" documents. They are not in any sense "pure" sources of data. Censuses and other public records count only certain things, not others. They count some things better than others, as in this instance they count infant and child deaths better than births. They reveal a society's particular system of classification. So they are not so much mirrors of reality as they are filters, or "collective representations," as Émile Durkheim might put it. It is just those images and collective representations—in this case, of the child and of child death—that I am after. How are the records kept? What events are kept track of? What is thought hardly worth noting or counting at all? And what can this tell us about the collective invisibility of women and children in particular?

Those relatives who arrive at the *cartório civil* to register the death of a child in Bom Jesus are briefly interviewed by Dona Leona according to the following formula. They are asked to testify, "on their honor," to the time and date of the death, the place of death (usually the home address), the sex of the child, the child's "color," the name of the child's mother and father, the father's (but not the mother's) birthplace, the father's (but not the mother's) profession, and the name of the cemetery. The reporting relative is then asked to sign the form or to affix his or her mark (an *X*), and two other individuals are asked to testify as "witnesses" to the accuracy of the account.

What we can learn from this particular record is the following: the sex, age, name, and "race" of the child; the marital status of the parents; the neighborhood or street in which they live; the father's occupation; and the place of death. In those few instances where the child died in the hospital and was also issued a death certificate, Dona Leona noted the name of the attending physician and his or her diagnosis.

While copying the birth and child death data at the registry office, I was able to observe many interactions between Dona Leona and the people of Bom Jesus, especially the poor of the Alto, who appeared each day to register the death of a child. Most often it was the father of the child who appeared, but occasionally it was a grandmother, a grandfather, an aunt, a godparent, or even an older sibling. Mothers, however, never appeared in the *cartório* to register the death of one of their own children. The registration and burial of the child usually took place within twelve to twenty-four hours of the child's death.

Dona Leona was generally distant and officious; if provoked, she could be gruff and dismissive, especially if the relative was uncertain of basic "details," such as the name of the child, the complete names of the child's parents, the marital status of the parents, or the exact time and location of the death. Many of these seemingly obvious and necessarily bureaucratic details were anathema to the people of the Alto and had little relevance to their everyday lives. "Name of the deceased?" Dona Leona snapped. And I saw a father turn anxiously to his sister-in-law to ask her, "Whatever *is* the name of our little Fiapo [a common nickname meaning little bit of nothing]?" Explaining where one lived in response to the bureaucratic question "Street and house number?" could be taxing. There were no official house numbers and only descriptive and informal nicknames for many of the dirt paths and hillside ledges on which *moradores* had built their homes on the Alto do Cruzeiro. Living arrangements were often informal, and couples frequently did not know each other's surnames. On one occasion, when a father could not produce the full name of his common-law wife, his *compadre* whispered to him, "Well, just let it be Araújo da Silva, then" and so "married" the couple on the spot. Dona Leona was not amused by these "lapses" of memory in her clients, and she was not above giving them an occasional dressing down.

As the end of the day approached, Dona Leona could be testy; her work kept her busy, and she liked to have her books in order by 4 P.M. so that she could go home early. Those who rushed in at the last minute, as did Dona Aparecida of the Rua dos Magos, could face an impassive and bureaucratic wall of resistance. Aparecida had just run from the Barbosa Hospital at one end of Bom Jesus to the *cartório civil* at the other end to register the death of a premature grandson who had been born and who had died earlier that day. Her daughter, the infant's mother, was doing poorly in the hospital and had begun to hemorrhage. The baby's father was away working on a distant plantation and knew nothing of the events that had transpired. It fell to Aparecida to bury her grandson, but in her anxiety over her deathly ill daughter, she had forgotten to register the death earlier in the day, and now Seu Moacir refused to give her the little coffin until she had done so.

"But where is the marriage certificate?" Dona Leona inquired, as the older woman attempted to register her grandson as the "legitimate" son of her daughter and son-in-law. "And how would I know where my daughter keeps such things?" replied the grandmother, who was sent away in search of the document and told to return with it the following day. And so Dona Leona got to go home early, as usual. The infant, meanwhile, wrapped in its *mortália* (winding sheet), lay overnight in a hospital storage room that served on occasion as a morgue for indigent patients.

Dona Leona tried hard to run a tight ship, but she was often frustrated. "Color of the deceased?" she asked. And here the relatives were often puzzled. Some fathers pointed to their own skin, saying, "Well, she was my child." In other words, "Judge for yourself, if you wish." Another young father, when asked the color of his deceased four-month-old infant, replied, "It was just a baby—it didn't have any color yet." Usually Dona Leona simply designated the color of the infant based on a local cultural category: poor equaled "brown." She never asked the parents to supply the missing "cause of death," however. In the absence of an official death certificate and a medical diagnosis, there was, she said, no way to know, and she was content to leave that space blank on most of the forms.

The state, then—represented in the personages of minor civil servants such as Moacir and Dona Leona—contributes to the routinization and normalization of child death by its implacable opacity, its refusal to comprehend, and its consequent inability to act responsively to the human suffering that presents itself. Bureaucrats and civil servants respond to pain and difference with a studied indifference—*la belle indifférence*. Normally, this is expressed in the bureaucracy's "deaf ear," its interminable off-putting delays and postponements, its failure to note the dire consequences of its indecisiveness. But there is another side to bureaucratic indifference that is more

Table 7.1. Births and deaths in Bom Jesus da Mata.

	1965	*[1977][a]*	*[1978][a]*	*1985*	*1987*	*1989 (Jan.–July)*
All deaths (adults and children)	1,117	N/A	N/A	507	444	327
All births	760	[761]	[896]	951	722	N/A
Infant/child deaths (< 5 years)	497	N/A	N/A	214	170	162
Infant deaths (< 1 year)	375	[311]	[320]	165	152	155
Child deaths (1–5 years)	122	N/A	N/A	49	18	7
IMR	.493	[.409]	[.357]	.174	.211	N/A

Source: *Cartório civil*, Bom Jesus, based on computations from individual records of each death.
[a]Summary data, supplied by local IBGE office.

characteristic in this instance: the rapid dispatch. It conveys that nothing of any consequence, nothing worth noting, has really taken place. Two or three minutes to "process" each dead infant or child should suffice.

"But look at this," I spoke (out of turn) from my perusal of the death registry books in the *cartório* on one occasion. "Here is the name of a woman on the Rua dos Índios of the Alto do Cruzeiro who has lost three small children within the space of a few months. What do you think could be going on? Shouldn't someone look into it?"

"I wouldn't know," Dona Leona replied coolly. "My job is only to record the dead, not to hold an inquest once they're gone."

I was sufficiently abashed to go quietly back to my "clerking" of the records, scratching away in my copybooks, a modern-day Bartleby the Scrivener, refusing to leave where I was decidedly unwanted and just barely tolerated. Table 7.1 shows what my clerking revealed.

In 1965 a total of 497 child deaths (of those born live through age five) were recorded. Three hundred seventy-five, or 78 percent, of the deaths were of infants in the first twelve months of life. These infant deaths came from a total of 760 recorded births for 1965, a figure that included all children born in that year and registered in that year or born in 1965 but registered in subsequent years. Even if we assume that some born in that year escaped registration altogether, 1965 (with an infant mortality rate of 493/1,000 live births) was a year the bells of Nossa Senhora das Dores tolled incessantly. And when they tolled, it was for the "holy innocents" of Bom

Jesus and its rural surrounds, the most immediate victims of Brazil's "quiet and bloodless" military coup. Quiet, indeed, but perhaps not quite so bloodless after all. Of all deaths in Bom Jesus in 1965, 44.5 percent were of children younger than five years. The high proportion of child to adult deaths did not change radically over the next two decades.

With the establishment of mandatory birth registration in the mid-1970s, birth statistics are now more reliable than previously, although they are still incomplete. Twenty years later, in 1985, 214 child deaths were registered, of which 165, or 77 percent, were of infants younger than one from a total of 951 registered births. The infant mortality rate had decreased significantly to 174/1,000 live births. Of all registered deaths in Bom Jesus in 1985, 42.2 percent were of children younger than five years.

In 1987, the final complete year studied, 170 infant and child deaths were registered. Of these, 152, or 89.4 percent, were infants, with 722 recorded live births for the *município* that year. Of all deaths that year, 38.2 percent were of children younger than five years. When I returned again in 1989 I was able to review births and deaths during the first seven months of that year (January–July). The data indicated a significant upward turn in child, and especially infant, deaths. There were already 162 infant and child deaths recorded, of which 96 percent were infants. Meanwhile, the proportion of child to adult deaths had increased to 49.5 percent—that is, in 1989 half of all deaths in the *município* were of babies in their first year of life.

In all, the data indicate a decisive decline in infant and child mortality in the twenty-year period 1965–1985, beginning from an apparent 49 percent infant mortality in 1965. It must be recalled, however, that 1965 was by all accounts an extraordinary year for child deaths, the year of the great die out of Alto babies, resulting from a severe drought combined with the economic and social chaos occasioned by the military coup d'état of March 1964. Many social programs that were put into place by the state government led by the progressive, socialist governor, Miguel Arraes, were dismantled in 1965, with immediate and disastrous effects on the poorest populations. The high infant mortality was the most immediate indicator of the disruptions caused by the political and economic upheavals.

In these data we can begin to detect the modernization of child mortality pattern that I suggested for the nation as a whole—that is, the standardizing of child death within the first twelve months of life and its containment to the poorest and marginalized social classes. Although in the aggregate data for the three years that I personally surveyed 82.9 percent of all child deaths occurred in the first year of life and 57.7 percent in the first six months, in breaking down the figures, we can see that in the past few years *infant* death has come to replace *child* death almost completely. Between 1965 and 1989

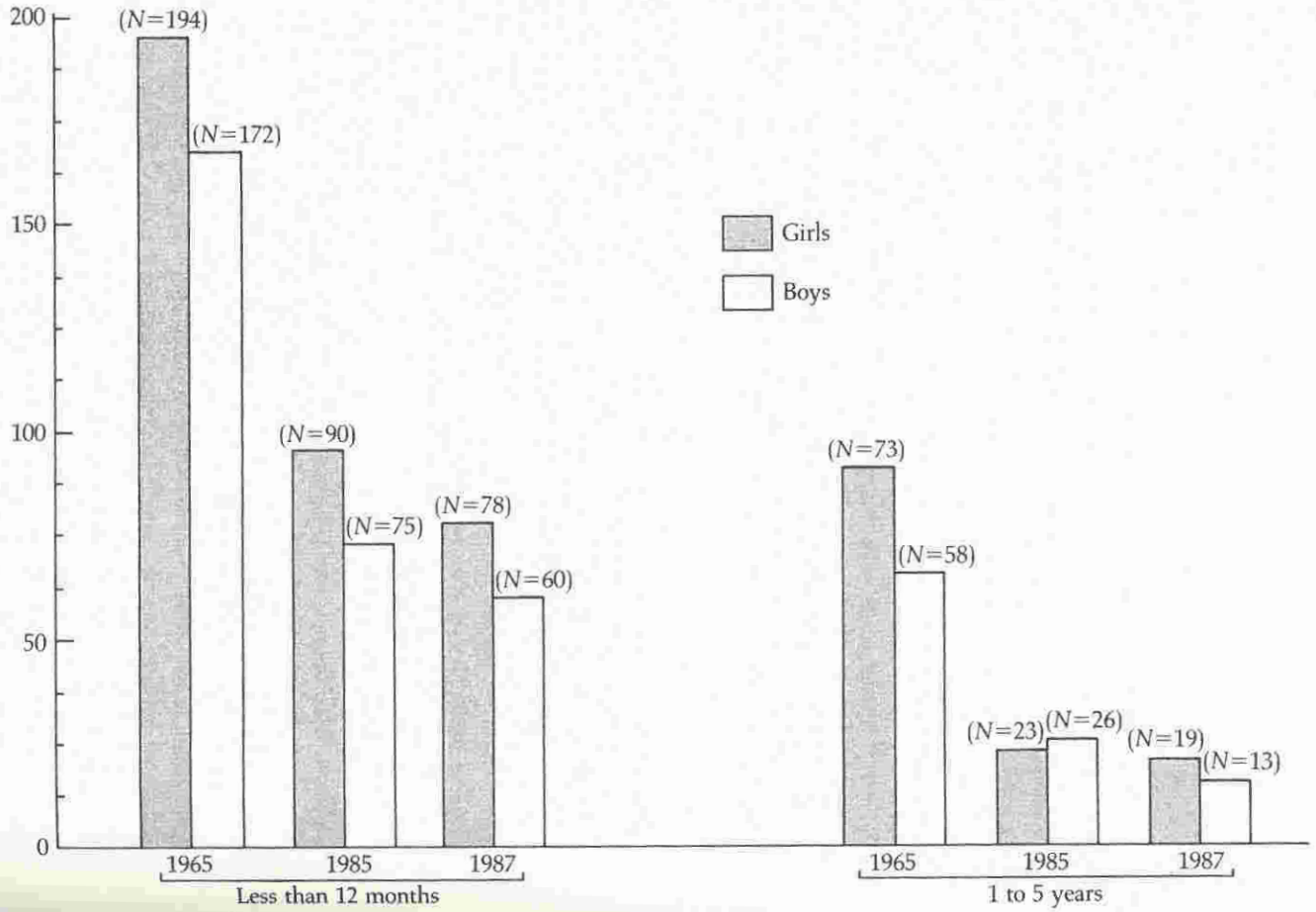
the percentage of child deaths after the first year of life through five years declined from 25 percent to 4 percent. By 1989, 96 percent of all child deaths were of infants in the first twelve months of life. Today, older babies and toddlers of Bom Jesus are reasonably safe from chaotic and premature death. At the close of the decade of the 1980s, it was safe to say (as the popular culture indeed dictated) that if a child survived her or his first year, the family could bring out the baptismal clothes and celebrate a formal, rather than a hurried, last-minute, christening: the child would live. Figure 7.1 demonstrates the changing pattern of child mortality in Bom Jesus.

If we take together all 881 cases of infant and child death culled from the records of the *cartório civil* for the years 1965, 1985, and 1987, some significant features of child mortality in the *município* emerge. Male infants and babies are at a slightly greater risk of mortality (54.1 percent male, 45.9 percent female overall) immediately postpartum (a predictable and fairly universal phenomenon [see Waldron 1983]), but even more so in the first five months of life, when there is some evidence of maternal influence and selectiveness in the relative survivability of babies. Figures 7.2, 7.3, and 7.4 show the relative disadvantage of male children at various ages.

Seasonality is a factor in child mortality in Bom Jesus. "There is a time to be born and a time to die," wrote the authors of Ecclesiastes, and in Bom Jesus da Mata children die "in season," particularly in February and June, the months of *festa* as well as the beginning and end, respectively, of the season of unemployment among the sugarcane cutters. By mid-February the harvest season is completed and the vast majority of Alto men are out of work. February is also the end of summer and a time of drought and thirst when the water supply is low and tempers are short. But February is also the time of Brazilian *carnaval*, when thoughts turn to the sensual, the festive, the imaginary, and the fantastic. It is, wrote Roberto da Matta (1983), "a time of forgetting." Indeed, among those mundane and bothersome things that are "forgotten" during this time of laughter and license are the needs of sick and vulnerable infants and small babies, some of whom are "lost" in the festive shuffle. And so the municipal coffin maker's folk theory is substantiated in the data.

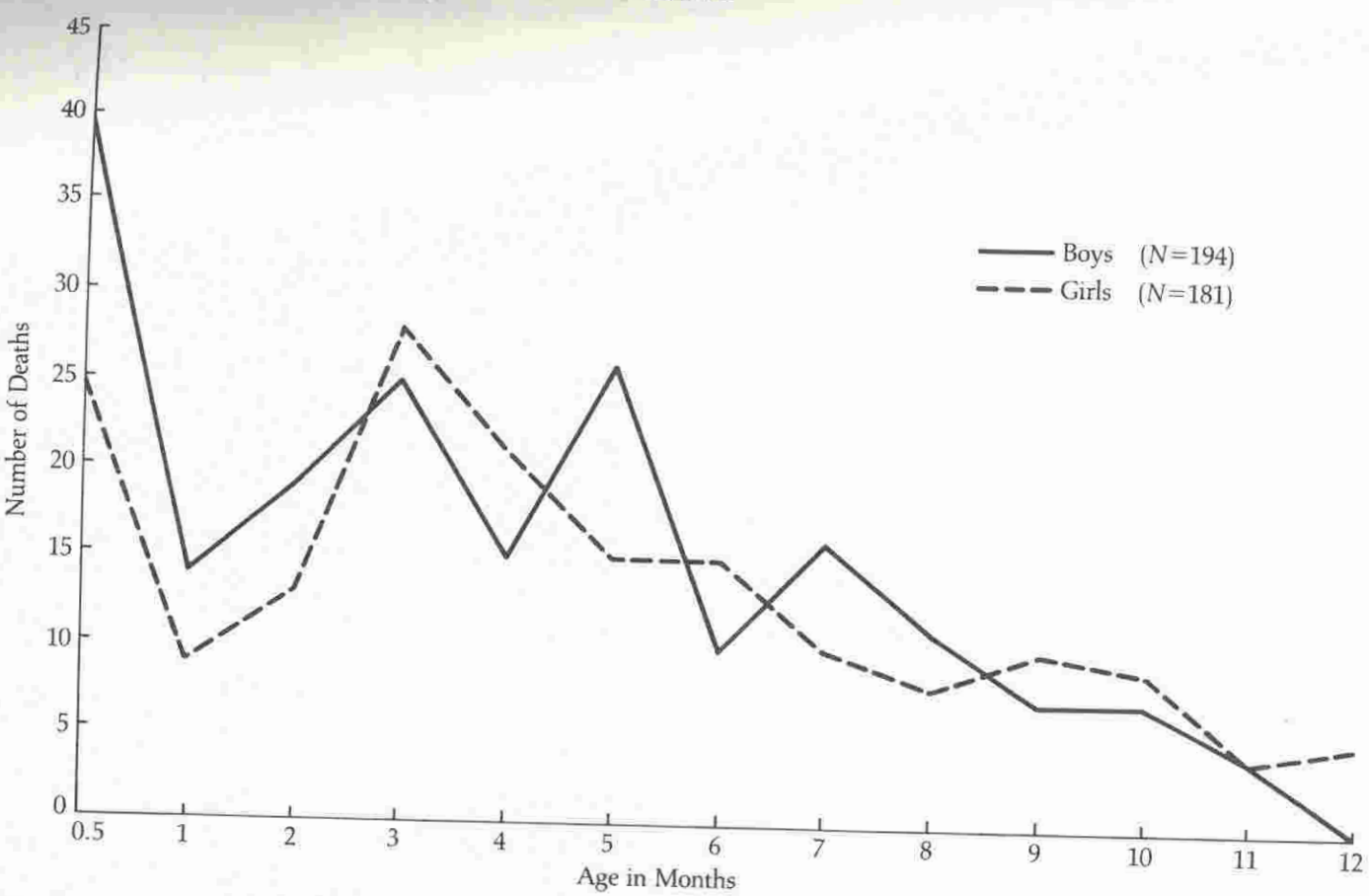
June is the month of winter holidays—the *festas juninas* of Saints Antônio, João, Pedro, and Paulo. Schools are closed, and the main streets are decorated for block parties, street dancing, and enactments of folk pageants such as the *casamento matuto* (the country yokel's shotgun wedding). As in February, during this festive cycle, too, the mayor's office, Justice Department, hospital, health posts, and clinics operate at low gear. It is not a good time to be sick or needy, child or adult alike. If the months of December through February assault small bodies with the relentless heat of summer

Fig. 7.1. Registered deaths by sex and by age group in Bom Jesus da Mata, 1965, 1985, 1987.



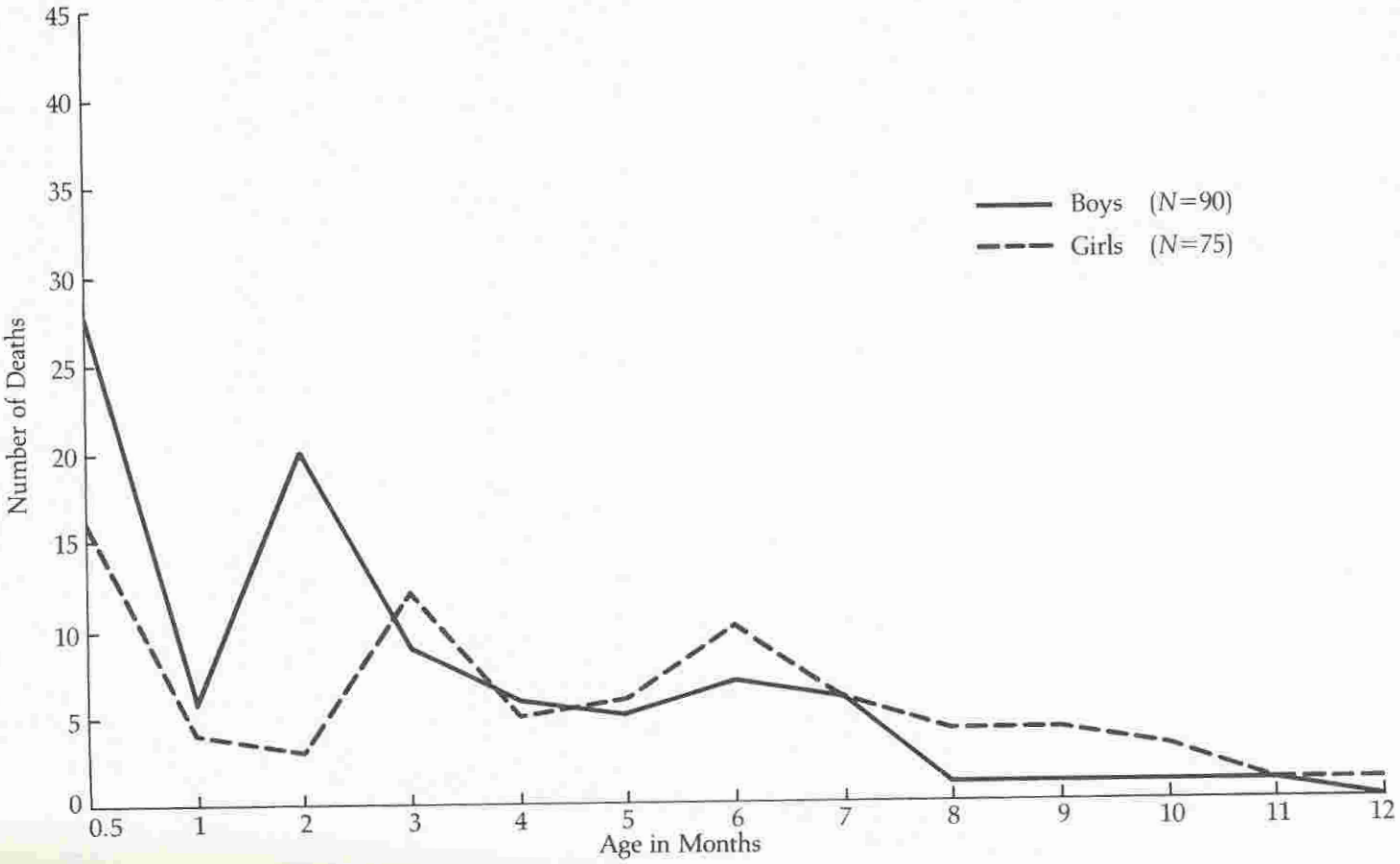
Source: *Cartório civil*, Bom Jesus da Mata.

Fig. 7.2. Registered deaths in the first year for boys and girls, 1965.



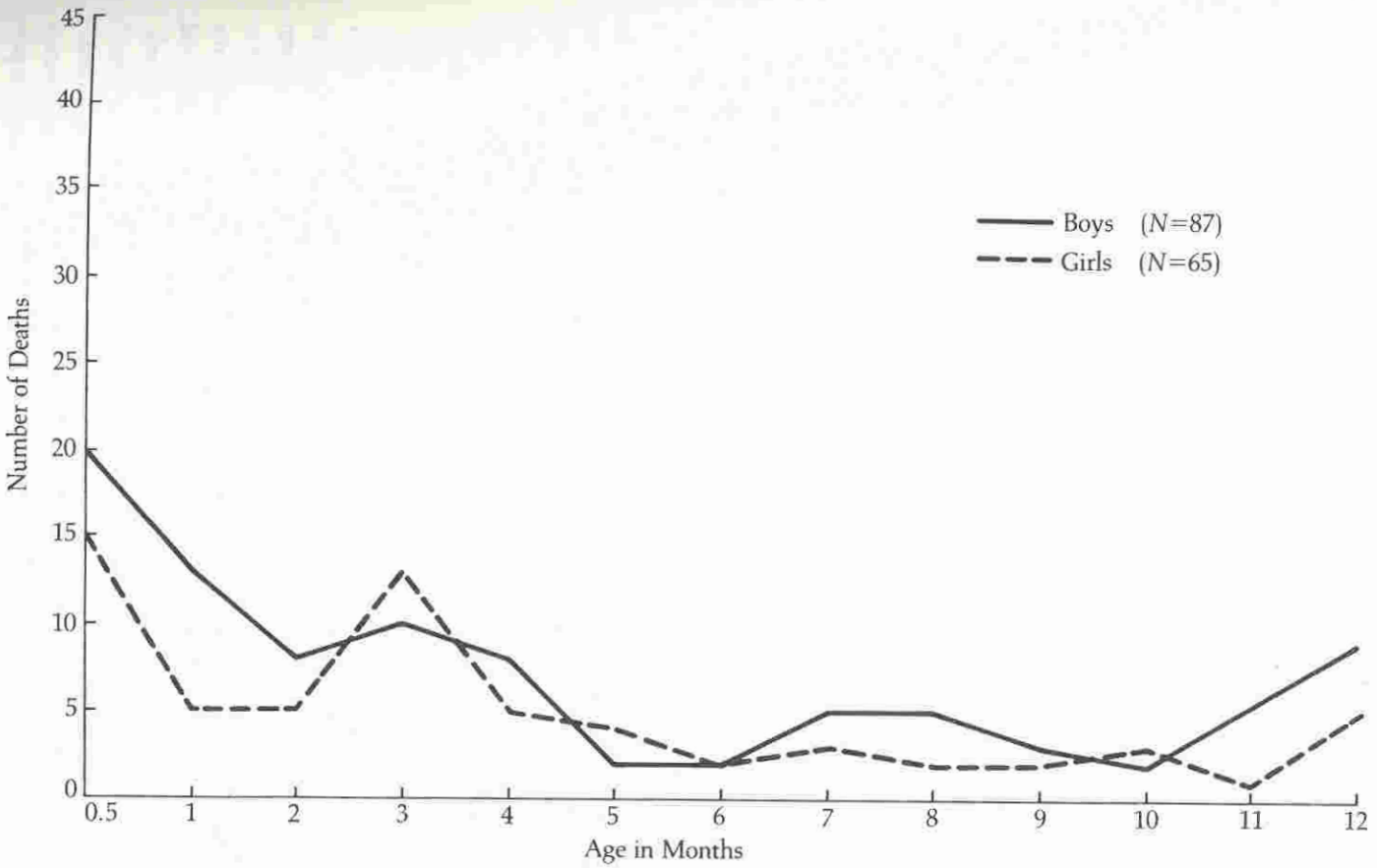
Source: *Cartório civil*, Bom Jesus da Mata.

Fig. 7.3. Registered deaths in the first year for boys and girls, 1985.



Source: *Cartório civil*, Bom Jesus da Mata.

Fig. 7.4. Registered deaths in the first year for boys and girls, 1987.



Source: *Cartório civil*, Bom Jesus da Mata.

and the drought, June marks the start of the winter downpours that turn the Alto into a running cesspool of backed-up latrines and wet garbage that carry in their wake new cases of infant and adult diarrheas. In addition, winter brings the flus and respiratory ailments to which so many Alto infants succumb.

From the birth and death records kept at the *cartório civil* we can also learn something about the families and households most prone to losing one or more children. Of the parents who lost children in the years sampled, 32.3 percent were from the rural districts of the *município*, living on large plantation estates and *fazendas*, while 67.6 percent were from the town of Bom Jesus itself. Of these, close to 87 percent of the child deaths occurred in the poor and marginal *bairros* of Bom Jesus, especially in the Alto do Cruzeiro, the largest and poorest of the peripheral neighborhoods of Bom Jesus.

Overall, 38 percent of the babies were registered as "white" and 62 percent as "brown," "mulatto," or "black." The proportion of white to brown infant and child deaths decreased precipitously between 1965 and the 1980s. This is a feature of the modernization of child mortality—its containment to the poor *favelas*, shantytowns, and rural districts where to be poor means, culturally speaking, to be brown or black. Of the 881 cases of child death, 38.3 percent of the parents were legally married, while 30.4 percent of the parents appeared to be living together in what *moradores* referred to as conjugal *amizade*, "friendship." In 30.9 percent of the child deaths, the mother was listed as single, and no father was identified. In those 496 instances in which the father's profession was listed, the majority, 328 (66 percent), listed agriculture as their profession. The remainder were scattered among the various trades and crafts of Bom Jesus: construction, carpentry, pottery, hammock making, and shoemaking. Fifty-nine fathers were listed as menial laborers and "servants."

Perhaps the most persistent feature of child mortality in Bom Jesus is its privacy and anonymity: 79.2 percent of all infant and child deaths occur at home without medical intervention or observation. Consequently, the causes of death for the vast majority of children younger than five in Bom Jesus da Mata remain unknown. But even among the 175 babies who died in the local hospital in 1965, 1985, 1987, 16 of these left the hospital for the cemetery without a cause of death listed on their official *óbitos*. A child could even die in the local hospital "unobserved." Of the 159 children with a medical diagnosis on their *atestados de óbitos*, the most frequent "cause of death" given (34.8 percent) was simply "cessation of heart beat and respiration." They died, one might suppose, of having lived. Dehydration was the next most common diagnosis (22.2 percent), followed by prematurity (14.8 percent). A dozen children died of falls, bruises, or blows to the body; *pancada na cabeça*, a "strike or blow to the head," was the commonest of these. Several carried the extraordinary medical opinion that the cause of death was acute infantile suffering. Only 3.4 percent of the deaths of infants and children in the hospital were attributed to acute or severe malnutrition, while only 1.7 percent of the infant and child deaths in the hospital were attributed to infant diarrhea. One wonders, amid the sea of froth and brine that carried away the infants and babies of the Alto do Cruzeiro, what kind of professional prudery it was that "failed to see" what every mother of the Alto do Cruzeiro knew without ever being told. "They die," said one woman going straight to the heart of the matter, "because their bodies turn to water."

On my return in 1989 I checked to see if the proportion of hospital to home, public to private, deaths had changed. In the first half of 1989 fully 71 percent of all child deaths still occurred at home; 29 percent occurred in the hospital, which represented only a slight increase in medically treated and observed child deaths. But the indifference to medical diagnosis had not changed: "Respiration stopped, heart stopped" remained the preferred medical diagnosis in cases of child death.

Reproductive Histories: Alto Women's Voices

My search to understand the extent and meaning of child mortality was not, however, contained to the public records, such as they were. If anyone in Bom Jesus understood the phenomenon, it was most certainly the women of the Alto do Cruzeiro themselves. To track their reproductive life histories, I began in 1982 with a sample of seventy-two Alto women; the sample eventually encompassed more than one hundred Alto women, and in 1989 I added a small sample of Alto men. Insofar as I have previously published the findings of my initial survey (1984, 1985), I reproduce here only the most significant features of Alto fertility and mortality, as gleaned from the personal reproductive histories, and I correct some of my initial findings in light of subsequent research.

My initial sample of Alto women was an opportunistic one, comprising the first women to volunteer following an open meeting in the abandoned creche of the shantytown. Many more women volunteered for the long interview than I could possibly have completed during a field trip that was also concerned with time-consuming general ethnographic work in the community. At the original meeting with Alto women I explained in general terms the purpose of the reproductive interviews. Because my work on the Alto during the 1960s had focused on maternal and child health, the current research agenda was both familiar to, and compatible with, the interests of the women themselves. My only criterion for inclusion in the study was

that the woman had been pregnant at least once. Consequently, the women spanned three generations of mothers, ranging in age from nineteen to seventy-six. This gave me the opportunity to compare reproductive patterns over time.

Throughout the process of eliciting women's life and reproductive stories, I was aided by a talented assistant, Irene Lopes da Silva, a resident of the Alto do Cruzeiro and one of the original founders of the shantytown association and the creche. She was among the first women I sought out on my return in 1982. Although both ill from a recent miscarriage and dispirited on the morning I found her "camping out" as a squatter amid the dilapidated and abandoned public washstands of the creche that she had helped to build in the 1960s, Irene was up from her hammock on the following day and ready to work as my primary research assistant. By 1988 Irene was able to conduct ethnographic interviews and take elliptical and somewhat idiosyncratic field-notes on her own. I wager that Irene, with less than three years of formal schooling, is as good a descriptive ethnographer as many graduate schools of anthropology have produced. She has the gift of the sociological imagination as she intuitively links the individual "worries" and "troubles" of Alto women to larger social, economic ills and to the political issues confronting contemporary Brazil. Her curiosity about human life, will, and action is enormous, and I am much in her debt with respect to understanding Alto women's moral choices and dilemmas.

The questions I put to Alto women were not only concerned with patterns of fertility and mortality but also with issues of value, choice, and meaning. I wanted to understand what infant and child death meant to Alto women and how they explained and interpreted their lives and their actions as women, wives and lovers, and mothers. What were the effects of scarcity and deprivation on women's abilities to nurture, "hold," love, and hope. What kinds of resilience did they have, and how were their "survivor" values and practices passed on to their children? For it was as survivors that many Alto women saw themselves. "*Vingaremos*!" they said, meaning that they did *and* will continue to survive despite the odds and that they would "take vengeance" on a life that had often treated them so unfairly.

A profile of the "average" woman in my Alto sample could read as follows: she was born on an *engenho*, where she grew up at the foot of the sugarcane; she attended primary school briefly, and although she could do sums with great facility, she could not read or write. Only a third of the women could, and with some difficulty, sign their names, a skill that until very recently determined one's legal and civil status as a voting citizen (an *eleitor*) or a nonvoting citizen. It was a biographical note that was carefully recorded on each person's death certificate.

After establishing a household with a man—often by "running off" with him (*fugindo*) and less often by having a civil or religious ceremony—she moved with her companion and children several times before finally coming to settle in town. Her husband or current companion was often described as "good" but *meio-fraco*, weak-poor, unskilled, unemployed, or, worse, sickly and dependent or as a *cachaceiro*, a "rum drinker." They might have broken up and come back together several times. Although most of the seventy-two women first interviewed in 1982 valued monogamy and marriage, less than half of them, thirty-two (or 44 percent), had a stable, long-term union that was recognized by church, state, or both. The remaining forty women (56 percent) described themselves as *solteiras*, as "single women," although eighteen of these were at the time of the interview living in a relatively stable relationship with one man, while fifteen had lived off and on in a series of relatively unstable relationships with different men. The remaining seven women described themselves as recently "abandoned." Nonetheless, Alto women were philosophical: "Terrible without them, but even worse with them," they often said of the "weaker" sex.

"Two men already left me," said Little Irene. "Do you think I need another? Nothing doing! Men are so jealous; I don't need any of these bandits around here. When my old man came around to 'visit' me the other night, I laughed in his face. I take whatever money he wants to give me for the children, but I won't let him pass the night with me as long as he has another little 'piece' around. What some of the younger women haven't learned yet is that marriage is a kind of slavery."

Edilene, aged forty-seven, agreed: "I prefer my children to my husbands. The husbands were all drunkards, and when they drank they would abuse me. But I would just up and leave them. It's much better to be alone than to take abuse. Some of it is my own fault; the men I arranged for myself were pretty but worthless. I have no 'knack' for men, so it's better that I remain single after all."

Nonetheless, there were many men and women on the Alto who shared a deep and lasting affection for each other, as did Black Irene and her second "husband," who came to live with her after Irene had already given birth to a child by another man. They lived happily together (*amigados*) for thirteen years. She had seven children by him, and then just before he died, they married. "Ours was a good marriage," Black Irene explained, soon after her husband was murdered. "I always wanted to be the one to die first. We weren't like those brutal couples that fight all the time, so that when one finally dies the other one brushes his hands and says, 'Good riddance.' No, we lived together in harmony. Whatever Seu Manoel [Irene always referred to her late husband formally and by title] wanted I wanted, and vice versa.

Whatever time of day Seu Manoel would come home, I would have his dinner ready. I never complained if he was late. And if I was late with his dinner, he never complained either. He would just sit patiently and wait until I called him to the table. A milder man than Seu Manoel would be hard to find here on the Alto do Cruzeiro. With love in your heart you can face almost anything."

The "average" Alto woman worked, at least part-time, taking in laundry or doing other domestic work for the wealthy families of Bom Jesus, clearing the fields of the smaller estates and sugar plantations, doing piecework for the local hammock industry, or selling produce at the weekly *feira*. If there was a man in the household, he was most likely a worker in the cane who was seasonally employed and unemployed in turn. Or he might have worked as a municipal laborer or street cleaner, earning only a fraction of a single minimum wage. If he was extremely lucky, he landed a job in a shoe or a textile factory. If he was unlucky, like Seu Biu de Papião, he collected paper and cardboard from the streets and garbage dumps for resale. A wife and husband's combined weekly income in 1982 averaged twenty-four dollars, or one and a half minimum salaries. This had dropped to only sixteen dollars a week in 1987, plunging the vast majority of Alto households from just ordinary to absolute misery. Women counted the members of the nuclear family from above and below: the dead angel-babies and wandering souls above along with the *desgraçados*, the unlucky and the accursed ones, still struggling below.

The seventy-two women in the sample reported a staggering 688 pregnancies among them, staggering in that half the women had yet to complete their reproductive lives. The "average" woman in the sample (see Table 7.2) had experienced 9.5 pregnancies and had 4.2 living children. She had 1.6 miscarriages, abortions, or stillbirths, and she had lost 3.6 small children (2.9 of those infants) among those born live to her.

I am often asked how "dependable" these women were with respect to their accuracy and truthfulness in reporting on their reproductive lives. Most but not all of the interviews were conducted in the creche and in a public context. The details of fertility were not, in any event, a private matter on the Alto do Cruzeiro, with the exception of a certain delicacy around young, especially male, children. Pregnancy, abortion, stillbirths, and child deaths were common topics of gossip and conversation among Alto women, and at any given interview session a woman might have been accompanied to the creche by her mother, adult daughters, siblings, or female neighbors. Together they corroborated the particular woman's history, serving as one another's memory checks and guides. The events of pregnancy, birth, and child death were a vital part of women's culture,

Table 7.2. Reproductive histories of Alto women.

	Total Number	*Number per Woman*
Pregnancies	688	9.5
Children surviving to age 5	308[a]	4.2
Miscarriages/abortions	93	1.3
Stillbirths	16	.3
Infant deaths (birth – 1 year)	208	2.9
Child deaths (1+ – 5 years)	47	.7

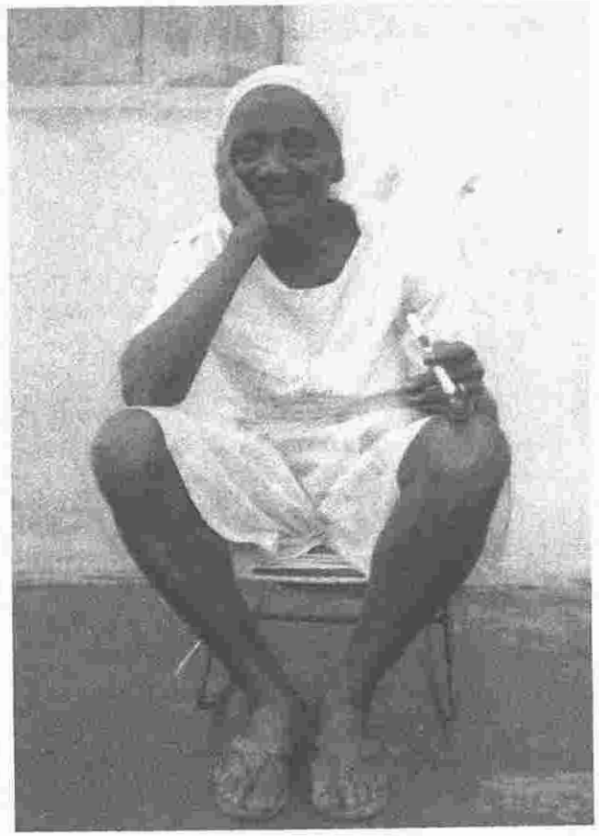
Source: Anthropological interviews of three generations of Alto women who gave birth between the 1930s and the 1980s.

Note: $N = 72$ women, ages 19–76, with a median age of 39 years.

[a]Minus 16 deaths of older children, 6–17 years.

transmitted from mother to daughter, so that adult daughters could recite their own mother's full reproductive history, including abortions, miscarriages, and deaths.

Although every woman could recall, without prompting, the exact number of pregnancies she had had (as a point of female honor and pride), a few of the older women could not recall the names of all their babies who died in infancy or the circumstances surrounding each death. There was a tendency for all the women to "round off" the ages of their children at death. If a child died as an infant, she or he tended to die at "two weeks," "one month," or "three months." A somewhat disproportionate number of Alto babies were said to have died at "three months," which was, I suspect, a convention invoked by mothers whenever an infant died quite young without any signs of teeth but not a newborn. There were certain mnemonic devices that women used to aid them in reconstructing what were sometimes awesome reproductive histories with as many as twenty pregnancies to account for. The women always began from the firstborn to the last born, and they used the formula *encostado de*, as when Zulaide explained that Zé Antônio came "leaning up against" Maria das Graças, and "pressing up against" Zé came Marivalva, and so on. I was impressed that even though Alto infants might not have been given names during their brief lives, they were always given Christian names at death, so that no Alto infant meets his or her Maker unnamed or anonymously. Insofar as newborns were frequently named after a preceding sibling who had died, mothers would sometimes recall their pregnancies in this fashion: "After Zulaide there was a long 'line' of Claudios—four in a row—but only the last one lived long enough to know his own name."



Interviewing Alto women: "My story is a long one; do you mind if I smoke?"

Perhaps a dozen women came to the interview carrying a small box or a tin can in which they had carefully stored a paper recording the birth or baptismal dates and names of each of their children. As most of the women were illiterate, they often had a *patroa* write these down as a favor. These folk "documents" were highly valued, so much so that when Dona Maria's wattle-and-daub house collapsed during the winter rains of 1987, she went back into the ruins the next day searching the broken clay pot fragments to find the handwritten record of her children's births and her widowed husband's hospital admission and funeral papers. In general the women were excellent reproductive historians, with one exception: I suspect that abortions, miscarriages, and stillbirths were underreported.

The reproductive histories of these women indicate certain patterns, some of which corroborate the statistics collected at the *cartório civil*. The period of

Table 7.3. Sex and age at death of Alto children.

	Male	*Female*	*Sex?*	*Total*		
Postpartum (1–15 days)	25	13	1	39		
15+ days – 3 months	51	37		88	173	
4 – 6 months	21	25		46	(67.8%)	208
6+ months – 1 year	14	21		35		(81.5%)
1+ – 2 years	11	10		21		
2+ – 5 years	10	16		26		
Total	132	122		255		

Source: Anthropological interviews of Alto women, aged 19 to 76, covering births between the 1930s and the 1980s.
Note: $N = 255$ deaths.

greatest risk for Alto babies is during the first twelve months of life, by which time 81.5 percent of all child deaths take place (see Table 7.3). But if one begins to break these aggregate data down by generation of Alto women (as I do in Table 7.4), we can see the same pattern I discussed earlier: infant death increasingly comes to predominate over child death in the younger generation of Alto mothers, those from nineteen to thirty-nine years of age. Bottle- and hand-feeding, in addition to patterns of mortal "selective neglect" directed at certain sickly and disadvantaged neonates and infants, both of which are products of disorderly and chaotic economic development, contribute disproportionately to the death of infants.

The reproductive histories of these impoverished women do not, however, suggest a particularly strong sex bias in child survival, although male infants are again shown to be more vulnerable immediately postpartum and up through the first three months of life. Perhaps reflecting this mortality pattern, Alto mothers were certainly more inclined to attribute a constitutional "weakness" (*fraqueza*) to their infant sons than to their infant daughters who did not survive. The absence of a "sex bias" in mortality is supported by the women, each of whom was questioned about ideal family size, sex preferences, and perceived differences in offspring.

Despite a strong and pervasive ideology of male dominance in Brazilian culture at large (although it was somewhat muted in the female-centered life of the shantytown), Alto women expressed no consistent pattern of sex preference, and all agreed that a woman would want to have a balance of sons and daughters. Boys were said to be "easy" to care for (once they survived infancy), relatively "independent" from a young age, and "useful" to the

household: small boys could be sent out "foraging" in the marketplace. They could earn small bits of cash carrying large baskets of produce on their heads for those able to pay, and little boys were not ashamed to beg (or even to steal) if necessity brought them to that. Besides, mothers added, boys were "entertaining"—what mother didn't take pride in her growing son's skill and dexterity with a soccer ball or, later, luck with young girls?

But daughters, too, were loved and highly valued—what mother didn't enjoy dressing up a little daughter or seeing her go off proudly to school with at least an initial determination to master her numbers and letters? Daughters were not only a mother's helpmate at home; when they grew up they often became a mother's closest, lifelong companion. And although distance, dissension, and alienation between mothers and adult daughters occurred on the Alto (as anywhere else in the world), it was considered both a tragedy and an aberration. In general, Alto women tended to stay in close proximity to their daughters so that small clusters of intergenerational, female-centered households built around the *matriz* (mother church but also the source or foundation of anything, in this instance, the first mother) were common on the Alto. Yes, a woman would be short-changed indeed not to have at least one *casal* (boy-girl couple) to rear in life.

With respect to birth order and survival, a clear pattern emerged. Among the generation of women older than forty with completed reproductive histories, the safest and most protected children were those who occupied the "middle ranks," neither the first nor the last child born. The deaths of Alto babies tended to come "in runs," often at the beginning or at the close of a woman's fertile years. In the earlier years the mother's youth and relative inexperience sometimes played a part in her children's deaths, as did the fact that many Alto women were single and unsupported during the earlier pregnancies and births. One older woman summed up a situation that was not altogether unique: "It looks like I had to lose the first half dozen so I could learn enough from my mistakes to save the second half dozen." A woman's advancing age, her mental and physical fatigue, and the drain on already limited resources made by several other older siblings contributed to the higher mortality risk of children born later. Some older women were explicit about their feelings toward the birth of their late-born and supernumerary children: they had had enough, the children were an unwanted burden, and the mothers were grateful when some of the children died in infancy. Nevertheless, the *caçula*, the last-born child to survive infancy, was generally loved and indulged by all: she had "adjourned" (*encerrou*) her mother's seemingly inevitable appointment with procreation, and that fact alone increased her value.

Before turning to a consideration of Alto mothers' perceptions of the

Table 7.4. Fertility, mortality, and ideal family size, by generation of Alto women.

	For Each Younger Woman[a]	*For Each Older Woman*[b]
Total pregnancies to date	6.6	12.4
Total living children	3.1	3.4
Total infant/child mortality	2.4	4.7
Abortion, miscarriage	1.1	4.3
Ideal family size	2.6	3.5

Source: Anthropological interviews of Alto women.

[a] $N = 36$. These women ranged in age from 19 to 39, with a median age of 30.

[b] $N = 36$. These women ranged in age from 40 to 76, with a median age of 50.

primary causes of infant and child death, I want to elaborate a bit more, by way of comparison across time (generational) and social space (class), on the social epidemiology of child mortality in Bom Jesus da Mata. As grim as the situation first appears, it is actually worse than I have described. The record of pregnancies, births, and infant deaths, as summarized in Tables 7.2 and 7.3, is incomplete. Many of the children reported as living at the time of the initial interview (1982) were deathly sick babies, and some of these have died in the interim. I did not try to follow up every woman in my original sample, but I maintained reasonably close contact with twenty-eight of the households, and on subsequent field trips between 1987 and 1989 I recorded an additional nine deaths from these households alone. A few of these additional child deaths were of new infants, whose births were not, of course, even recorded in the 1982 interviews.

Recall that only *half* of the original seventy-two women interviewed had completed their reproductive years. If we separate out the older cohort of women (median age = fifty years) from the younger women still in their childbearing years (median age = thirty years), we can see that the "average" Alto woman had experienced 12.4, not 9.5, pregnancies and 4.7, not 3.6, infant and child deaths (see Table 7.4). These figures are not, however, necessarily predictive of the younger generation of Alto women, whose reproductive thinking and practices differ somewhat from those of the older generation.

Generally in demography one cannot depend entirely on completed life courses across historical time; one must look closely at incomplete data as well. One way to do this is to compare the experience of first and second births between older women of completed fertility and younger women of

Table 7.5. Child mortality by generation of Alto mothers, first two pregnancies only.

	Younger Women[a]	*Older Women*[b]
Pregnancies	69[c]	72[d]
Miscarriages/stillbirths	5	4
Infant deaths (birth – 1 year)	27 (13 male, 14 female)	14 (10 male, 4 female)
Child deaths (1+ – 5 years)	0	7 (3 male, 4 female)
Older child deaths (5+ – 12 years)	1 (male)	2 (1 male, 1 female)
Living children	36 (24 male, 12 female)	46 (24 male, 22 female)

Source: Anthropological interviews of Alto women.
[a]N = 36. These women ranged in age from 19 to 39 .
[b]N = 36. These women ranged in age from 40 to 76.
[c]Three younger women had had only one pregnancy to date.
[d]One older woman gave birth to twins at her second delivery.

incomplete fertility. Table 7.5 demonstrates the extent to which things are getting worse for the younger generation of Alto women and their babies. The younger women already show an "excess" of ten deaths, or an additional 1.4 per woman, over the preceding generation of Alto mothers, this without us even knowing (or wanting to predict) how many others from among the younger generation's still-young infants and babies are yet to die in the months to come. Looking at these figures with their built-in question marks puts one in mind of Ebenezer Scrooge's last eerie visitation: the ghost of Christmas yet to come. And we wonder, along with Scrooge, whether the premonitions of deaths foretold in these incomplete figures represent what *must* happen or only what *may* come to pass.

Mortal Ills, Fated Deaths

Whereas doctors in the clinics and hospitals of Bom Jesus were unconcerned about properly diagnosing and recording the causes of infant and child deaths for the poor of Bom Jesus, Alto women readily shared with me their notions of the causes of childhood mortality. I posed the question in two ways. I asked a general question: "Why do so many babies die on the Alto do Cruzeiro?" Then in the course of recording personal reproductive histories, I asked each woman to tell me at length about the circumstances surrounding each child's death, including her perception of the baby's key symptoms, the

various steps she took to remedy the illness, and her understanding of the cause of the death. The two questions elicited very different answers.

In response to the general question on the incidence and causes of high child mortality in the community, Alto women were quick to reply with blanket condemnations of the hostile environment in which they and their children were forced to live and die. They responded:

"Our children die because we are poor and hungry."

"They die because the water we drink is filthy with germs."

"They die because we cannot afford to keep shoes on their feet."

"They die because we get worthless medical care."

"They die of neglect. Often we have to leave them alone in the house when we go to work. So you wash them, feed them, give them a pacifier, close the door, and say a prayer to the Virgin hoping that they will still be alive when you get home. Yes, they die of neglect [*à míngua*] but it's not due to a lack of goodwill toward our children. The problem isn't one of *vontade* [willingness] but one of *poder* [power or ability]."

When asked what it is that infants need most to survive the first and most precarious year of life, Alto women invariably answered that it was *food*, pure and simple: "Can it be that mothers of ten, twelve, even sixteen children don't know what a child needs to survive? Of course we know! Rich people's children have proper food. Our children are fed catch as catch can. Some days we have one ingredient for the baby's *mingau*, but we don't have the other. We may have the *farinha* but not the sugar. Or we have the sugar but not the powdered milk. And so we improvise. What else can we do?"

Another shook her head in perplexity: "I don't know why so many die. Some babies are born strong and healthy enough. Their stomachs when they are round and full give one such pleasure! But something is wrong with the food we give them. No matter how much we feed them, they lose their fat and turn into toothpicks. It makes one discouraged."

Still another could identify the exact problem and its remedy: "They die from the miserable *engano* [ruse] of *papa d'água*. Babies need food to live. Most older babies require at least two cans [four hundred grams each] of powdered milk a week. But people here can afford only one can, and so the babies are fed mostly on water. Soon their blood turns to water as well. Money would solve all our problems."

Many others agreed with her: "Here on the Alto there are a multitude of children who live in neglect, eating garbage that other people leave behind, sucking on banana skins and on orange peels. It's because their parents don't earn enough to feed them, and the only solution is to set the children out on the streets."

In short, Alto mothers gave highly politicized answers to the question of

child mortality in general, ones that stressed the external constraints on the ability to care for their offspring. But when these same women were asked to explain why any of *their own* children died, their answers were more clinical, and the causes of death were seen as more proximate, sometimes as *internal* to the child. Often the dead infant was judged as lacking a vital life force, his or her own "will" to live. And not a single Alto mother stated that hunger was the cause of death for any of her own children, although many of the dead babies were described as having "wasted" or "withered" away, "shriveled up," or "shrunk to nothing." In response to what may have caused a particular infant to "waste away," Alto women often replied that the baby was born with a "fragile," "nervous," or "weak" constitution. Hunger, it seemed, only killed Alto children in the abstract. It may kill *your* children (perhaps) but not any of mine. It may be that Alto mothers had to exercise a certain amount of denial because the alternative—the recognition that one's own child is slowly starving to death—is too painful or, given the role that mothers sometimes play in reducing food and liquid (see chapter 8), too rife with psychological conflict.

Alto women distinguished between child deaths viewed as "natural" (coming from God or from nature) and those suspected to be caused by sorcery, evil eye, and magical possession. They attributed most of their own children's deaths to natural causes, especially to communicable diseases. But the women also explained child deaths in terms of failures in proper nurturing, including disregard for the normal "lying-in" precautions for mother and newborn, mortal forms of neglect, and strong and passionate emotions. Table 7.6 gives a very condensed rendering of Alto mother's perceptions of the major pathogens affecting the lives of their offspring.

Alto mothers considered simple diarrhea the greatest single killer of their babies, carrying away 71 of the 255 children who died. But more "complex" and complicated forms of diarrhea were also implicated in the folk category *doença de criança*, "child sickness," and in some cases of *dentição* (teething illness), *gasto* (wasting illness), *susto* (fright sickness), and *fraqueza* (general weakness and debility). Were we to include all the folk pediatric diagnoses in which diarrhea was at least a secondary symptom, then as many as 189 of the 255 deaths, 74 percent of all infant and child deaths on the Alto, could be attributed to diarrhea. Mothers distinguished among many different subtypes (*qualidades*) of infant diarrhea (e.g., *intestino, quentura, barriga desmantelada*), based on color, consistency, smell, and force of the stools. Mothers saved the dirty nappies of their ailing babies to discuss the differential diagnoses with neighbors and elderly healing women of the Alto. In all, mothers recognized the severity of this disease in particular as a primary child-killer on the Alto. Among the other communicable diseases

Table 7.6. Causes of infant/child deaths (Alto mothers' explanations).

Cause	*Number*
Diarrhea/vomiting	71
Measles, smallpox, pneumonia, and other infectious childhood diseases	41
Doença de criança (doomed child syndrome, various subtypes)	39
Fraqueza (weakness, wasting)	37
Pasmo, susto (fright, shock, and other malignant emotions)	14
Dentição (malignant teething)	13
Diseases of skin, liver, blood	13
De repente (sudden death)	9
Mal trato (conscious, if unintentional, neglect)	6
Resguardo quebrado (lying-in precautions broken, violated)	5
Castigo (punishment, divine retribution)	4
Cause unknown to mother	3

Source: Anthropological interviews of Alto women.
Note: $N = 255$ deaths.

commonly cited by mothers were (in order of importance) measles, pneumonia and other respiratory ailments, infant jaundice, tetanus, fevers, whooping cough, smallpox, and other skin diseases and infections.

Underlying and uniting these diverse etiological notions were the same structural principles that informed Alto people's beliefs about the "nervous" body. Here, once again, life was conceptualized as a *luta*, a power "struggle" between large and small, strong and weak. Infants were born both "weak" and "hot"; their tiny systems were easily overwhelmed. Poor infants were already compromised in the womb. Born (as the women said) of prematurely aged fathers whose blood was "sick and wasted" and whose semen was "tired" and of mothers whose breasts offered blood and infection instead of rich milk, it is little wonder that Alto babies were described as born "already thirsty and starving in the womb," as "bruised and discolored," their "tongues swollen in their mouths."

The babies of the rich were described as coming into the world fat and fair and "greedy" for life. They emerged from the womb with a lusty cry. The babies of the Alto, "poor things," came out of the womb "like wet little birds, barely chirping" and with a "nausea" for food. "Our babies," I was often told, were "born already *wanting* to die." Although few Alto mothers could give me the accurate birth weights of their offspring, their descriptions of "skinny," "wasted," "pale," "quiet" newborns, infants who came into the

world with no *gosto* (taste) for life and no will to suckle, seemed very much the descriptions of preterm and low-birth-weight infants, babies all too aptly described by Alto women as born already "disadvantaged."[6]

What mothers expectantly looked for in their newborn infants were qualities that showed a readiness for the uphill struggle that was life. And so Alto mothers expressed a preference for those babies who evidenced early on the physical, psychological, and social characteristics of fighters and survivors. Active, quick, responsive, and playful infants were much preferred to quiet, docile, inactive infants, infants described as "dull," "listless," and spiritless. Although differences in infant temperament were believed to be innate, in a precarious environment such as the Alto parasitic infection, malnutrition, and dehydration reproduced these same traits in a great many infants. A particularly lethal form of negative feedback sometimes resulted when Alto mothers gradually withdrew from listless infants whose "passivity" was the result of hunger itself.

Conversely, for an Alto mother to say proudly of a child that she or he suffered many "crises" during the first year of life but "conquered" or "endured" the struggle was a mother's fondest testimony to some hidden inner strength or drive within the child. And so fat, resilient babies were described as having *força*, an innate charismatic power and strength. Many frail infants easily succumbed to death from teething because the innate *força* of the teeth straining against the soft gums holding the teeth captive overwhelmed the little body, making the infant vulnerable to any of several lethal and incurable child diseases. Perhaps the ethnopediatric illness *gasto* best captured the image of the beleaguered little body unable to resist powerful forces. In *gasto*, a fatal form of infant gastroenteritis, the infant's body offered no "resistance" and was quickly reduced to a hollow tube or sieve. Whatever went into the infant's mouth emerged directly in virulent bouts of vomiting and diarrhea. The infant was quickly "spent," "wasted," used up; her fight and her vital energy were gone.

Breasts, Bottles, and the Somatization of Scarcity

A fairly direct and positive correlation exists between infant survival and breast-feeding. A government-sponsored study in São Paulo found that among bottle-fed infants 32 percent were malnourished, compared with 9 percent of breast-fed infants. More recently, a team of researchers from the Department of Social Medicine at the Federal University of Pelotas studying the effects of mixed and bottle feeding on infant survival in a large case control study of two urban areas in the south of Brazil found that breast-fed infants who received even *supplemental* bottles of infant formula were at four times greater risk of dying than were breast-fed–only infants. The mortality risk jumped to fourteen times greater for infants who received no breast milk at all (Victora et al. 1989).

Bottle-fed infants are exposed to contaminated water; overly diluted powdered-milk formulas; unsterilized bottles, nipples, dishes, and spoons; and shortages in the supply and distribution of commercial milk. The evidence is so incontrovertible that a number of governments have taken unprecedented steps to end the practices of hand- and bottle-feeding infants. Legislation passed in Papua New Guinea in 1977 made baby bottles and plastic nipples available only by prescription. In 1980 the Nicaraguan Sandinista government implemented labor regulations that allowed mothers to breast-feed on the job, restricted the sale of infant milk formulas, and issued strong warnings on the labels of all infant food products.

Despite measures such as these, each generation of mothers in the Third World is less likely than the previous one to breast-feed offspring. This is especially true of rural migrants to urban areas, where wage labor and the work available to women are incompatible with breast-feeding. Consequently, powdered milk is one of the most readily available food commodities on the shelves of local *supermercados*, where it is sold in large, economical plastic sacks as well as in the more expensive, vitamin-enriched infant formula cans distributed by the Nestlé company. In Brazil the decline in breast-feeding has been precipitous; between 1940 and 1975 the percentage of babies breast-fed *for any length of time* fell from 96 percent to less than 40 percent (cited in Grant 1983:4). Since that time it has decreased even further.

Alto mothers of the present generation used the breast as an initial, and not very dependable, supplement to the staple infant food, *mingau*, that is offered to the newborn as early as the second day of life, following the cleansing herbal tea given to all infants immediately postpartum. Maternal colostrum is rejected as a "dirty" substance. It is manually extracted and thrown away. Most Alto mothers do offer the breast to their newborn by the third or fourth day postpartum but *always* in combination with, and usually following, *mingau*. Because hungry babies are quieted first with *mingau*, which is made of such a thick consistency that it is often finger-fed the newborn, it is not surprising that so many infants are "uninterested" in the breast when it is offered. Before long—usually in just a few days—mother's milk "fails." Alto infants are not put to the breast often enough, or hungrily enough, to suck vigorously and build up the mother's milk supply. And Alto mothers, like so many women elsewhere in the world (see Gussler & Briesemeister 1980), explain their failure to breast-feed in terms of "insufficient milk."

There is no doubt that the infant mortality rate of the Alto do Cruzeiro

could be reduced by half if *mingau* and powdered milk were replaced by breast-feeding. Nevertheless, nothing short of draconian measures could dislodge the practice of artificial infant feeding on the Alto do Cruzeiro. A national breast-feeding promotion campaign launched in Brazil in 1981 had little effect on the poor and illiterate women of the Alto, as elsewhere in Brazil. Meanwhile, the efforts of a local education campaign on the part of a number of concerned doctors, pharmacists, priests, and nuns have been poorly received by poor women, for whom breast-feeding is no longer really an option. Seu Wellington, a local pharmacist and social reformer, tried his best to turn the poor women of Bom Jesus away from their powdered-milk dependency. He sometimes used his pharmacy as a schoolroom or a pulpit. He explained, "I try to teach through analogy. I say to young mothers, 'Did you ever see a cat making *mingau* for its kittens, and yet most stray kittens on the Alto are better fed than your babies.' "

"*Mentira* [lie]!" interrupted a mother who was within earshot of our conversation. "Didn't I rear all mine with *mingau?*"

"And how many were there, Dona Maria?"

"Seven," she replied.

"And living today?"

"Three."

"Well, then, don't talk about rearing your children with *mingau*. You killed four of them with it, and the other three just managed to escape."

On another occasion an anxious young father arrived at the pharmacy with a sick and malnourished baby in search of medicines only to be scolded by Seu Wellington: "Who is the boss in your household? If *you* are, go home and break that blasted baby bottle and force that woman of yours to breast-feed!" When the man protested that his wife was ill and quite weak, that she had no strength to breast-feed, Seu Wellington countered, "That is just laziness in your woman. Go home immediately and show her who is the boss. Otherwise you will soon have a little angel to bury." The man left the pharmacy in a great state of perplexity and agitation, not knowing which way to turn.

The weekly purchase of the requisite four-hundred-gram tin of powdered milk for the household infant or baby is a chronic anxiety on the Alto do Cruzeiro today. Powdered milk (costing between two and three dollars per box, small plastic sack, or can) is the single most expensive food purchase made by most young Alto families, and it consumes about a fifth of their weekly income. It is little wonder that the quantity of milk purchased does not radically increase with the age and size of the current baby. The standard purchase remains one can or box per "baby," whether the baby is an infant or an eight-month-old. What differs is the "quality" of the milk purchased and

the extent to which it is diluted and starch filler is added. In general, the older the baby is, the lower is the quality of powdered milk purchased and the greater is the amount of manioc flour and sugar and water added to the *mingau*. It is believed necessary to give the newborn infant a "good start" in life by using the most expensive and "specialized" Nestlé milk formula, Nestogeno, first semester, recognized by its bright purple label with its picture of a sweetly sleeping, fat infant. After the first couple of months mothers may substitute Nestogeno for an "inferior," domestic brand of powdered whole or skimmed milk, at roughly half the cost of the Nestlé formula.

Even unschooled and illiterate mothers of the Alto make it their business to be well informed on the many different varieties of powdered milk sold locally—those designed for the neonate, those for babies in their first six months of life, those for babies in the second "semester" of their first year, and so on. There are many different tins and boxes of powdered milk, some fortified with iron and other vitamins and minerals, some without. I counted eleven different varieties of infant milk products available on the shelves of the central *supermercado* of Bom Jesus one morning in 1988: Nestogeno (recommended for the newborn), Nanon (for the baby aged six months to one year), Pelagron (fortified with iron), *Semilko* (a milk-substitute product with iron), *Ninho* (whole-milk powdered milk for the baby older than one), Nidex (a sugary, vitamin-enriched milk "supplement"). In addition to these (all Nestlé products), there were the following alternative brands of "baby milk": *Glória* (an inexpensive, skimmed, powdered-milk product manufactured by Fleischmann and Royal Industries); Molico, CIPE, Leite Componesa, and *Itambé* (Brazilian domestic brands of inexpensive, partly skimmed powdered milk). Some of these were produced at local sugar mills.

The array of "choices" was quite daunting, and the display of infant-formula powdered-milk tins and boxes took up a full aisle of the local supermarket, more than for any other food product. The "consumer consciousness" concerning the variety of commercial infant formulas in Bom Jesus was unlike anything I had ever seen. It was in a class by itself. The elderly midwife Norinha offered that she had simply "given up" because she no longer felt confident enough to instruct new mothers on the feeding of their infants: "Infant feeding is like a science today, and as for me, unfortunately, I am just a poor, illiterate woman."

It is too bad that the illiterate women of Bom Jesus cannot at least read the tiny, mandatory warnings printed on the brightly colored labels of all the canned Nestlé milk formulas (a concession by the industry to end the international boycott of its products that began in 1978). On the labels are such dire warnings as:



At the local market: Nestlé powdered milk, dozens of varieties.

"Before you use this milk, make sure you consult a physician."
"Powdered-milk formulas should be used only for healthy infants."
"Mother's milk is the best milk for infants."
"Remember to hold your infant when you feed him. An infant who feeds himself can easily strangle."
"By the age of six months you should supplement the milk formula with vitamin-rich cereals, eggs, fruits, and vegetables."
"If you mix this formula without boiling the water first, your baby can get very sick."
"Only make up one bottle of milk at a time if you do not have refrigeration."

Most of the warnings are inappropriate in any case. What Alto home has refrigeration? What Alto mother can consult a physician for advice on making up a *mingau*? Who can afford to boil water for every *mingau*? How can a mother hold her baby for each feeding when she must leave the baby in a hammock while she is away at work?

Alto mothers do recognize, however, that not all infants can tolerate powdered-milk *mingau*, and when a first crisis of diarrhea and vomiting occurs—often as early as the second week of life—women do experiment with different types of powdered milk, and they frequently vary the *mingau* recipe, substituting rice or corn starch for the offending manioc flour, adding more or less sugar, carefully filtering (if not boiling) the water, and so on. In very severe cases of pediatric diarrhea the infant is taken off *mingau* entirely and given only *soro* or an herbal tea to "calm" the intestines. But *no* mother tries to wean her sick infant from the bottle and *mingau* to the breast as an alternative.

The contemporary pattern of bottle- and hand-feeding differs markedly from the infant feeding practices of the older generation of Alto women, most of whom raised their infants and babies on rural plantations and *fazendas* where breast-feeding and woman's work were not so incompatible and where fresh cow's milk was plentiful and sometimes free to plantation workers and their families as part of the traditional terms of labor. Virtually all the older generation of Alto women breast-fed their firstborn children for a year or longer, though many then failed to breast-feed their latter-born children, allowing the older siblings to take over much of the rearing of their baby brothers and sisters. *Mingaus* and *papas* made from fresh, whole cow's milk were even in the previous generation an integral part of the infant's diet, but they were introduced later and after the nursing mother's milk supply was well established. But breast-feeding was not enough even then to curtail a high mortality, although the children of these older women died largely of communicable diseases, not of marasmic wasting and chronic

diarrhea/dehydration, the commerciogenic plague of the current generation of Alto babies.

The almost complete erosion of breast-feeding occurred during a very brief historical period. Twenty-five years ago, when I first lived on the Alto, women supplemented their breast milk with goat's milk *mingau*. The milk goats ran freely along the rocky ledges of the Alto, and they were raised by every other household. Today, with the erosion of the hillside, the milk goats have virtually disappeared. Only a half dozen women raise them, and tied up and contained to small sheds, they are pathetic and sickly looking for the most part. Their owners say that their young goats now die as "easily" as their babies. No creatures seem to be faring very well on the Alto today. And commercial powdered milk is the only substitute for the breast.

In the mid-1960s the only available powdered milk was supplied freely to Alto residents under the U.S. Food for Peace program. The women of the Alto were happy to receive the milk (which I myself distributed), but they used it only for cooking and for café au lait for themselves and hot milk drinks for their older children. They were "suspicious" of the "American milk" and fearful of giving it to their fragile infants and small babies. They did not believe that it was "real" milk at all; they thought it was made from "plants" (perhaps they meant soybeans), although others said it was manufactured from whitewash, while local "communists" spread the rumor that it was made from ground-up baby bones. I recall being offended by these rumors and "doing my best" to correct them. I assured Alto women that the powdered milk, mixed carefully with filtered or boiled water, could be given safely to small babies. Just as firmly the women informed me that the American milk gave their infants diarrhea, vomiting, and blindness. Much later I learned about the "night blindness" caused by vitamin A deficiency in babies fed exclusively on formulas made from powdered skimmed milk, such as the sacks of milk the U.S. government was pouring into Brazil and other underdeveloped nations at that time. Perhaps the communists were not so wrong after all. The Food for Peace program distributed powdered milk throughout the Northeast region during the 1960s and eventually fostered a powdered-milk dependency in the populace, which Nestlé and other companies took advantage of when the free distribution ended in the 1970s.

But why did the women of the Alto give up their original resistance to powdered milk? How were they turned into avid consumers of a product that they do not need, that they cannot afford to buy, and that contributes to the death of their children? If we assume that people behave according to the ground rules of a "minimal rationality" (see Mohanty 1989), we ought to look for the internal logic that informs the "choices" that people make.[7] But perhaps it is disingenuous to ask poor women who are often heads of

households with four or more dependent children, in addition to the current baby, what their infant feeding "choices" and "preferences" are. If they "prefer" to live at all, they must often work as temporary wage laborers and rear their infants as best they can and with whatever is at hand. In the desperate situation in which many Alto women find themselves, bottle-feeding is really the only possible "choice."

Black Irene breast-fed each of her children for a few months until she had to return to work. Her *patroas* would never allow her to enter their homes if they even suspected that she were lactating. "*Dá nojo* [It's disgusting]!" Irene exclaimed, gesticulating to indicate heavy, pendulous breasts dripping milk. One could not run the risk of suddenly having a wet blouse, she explained, in the middle of serving the family a meal: "It would make everyone lose their appetites."

In the transition from semisubsistence peasants to wage laborers, whereby food becomes a commody that is purchased, rather than grown on one's rented *roçado*, it does not take much of an imaginative leap to begin thinking of infant food as a prepackaged commodity as well. There are radical changes, too, in the relations between the sexes that further promote bottle-feeding. As marriages become less formal, more consensual, and more transitory in the shantytown (as opposed to the rural *vila*), the responsibilities of mothers and fathers to their offspring are altered, and the ways of symbolically establishing a child's legitimacy take on new forms, some of them harmful.

Shantytown households and families are "made up" through a creative form of bricolage in which we can think of a mother and her children as the stable core and husbands and fathers as detachable and circulating units. Consequently, the definition of a "husband" on the Alto do Cruzeiro is a functional one. A husband is the man who provides food for his woman and her children, regardless of whether he is living with them. Insofar as many men on the Alto provide (or try to provide) food to more than one household of women and children simultaneously, it is difficult to conceptualize the community as fundamentally monogamous, although monogamy remains the idealized norm. All Alto women—especially those who are not living with the father of their newborns—both hope and expect the putative father to recognize and "claim" his offspring by means of a highly symbolic transaction. That is, within a few days of the birth of the baby, the father is expected to provide the infant her or his first weeks' supply of expensive Nestogeno. The definition of father on the Alto do Cruzeiro is the man who arrives at least once a week bearing the prestigious purple-labeled can of Nestlé or, when relations are strained, who has the can of milk sent to the household through a friend or intermediary. In fact, this is how a woman's



Papa: baby's "milk."

current lover and father of her child is often greeted. As he sheepishly enters the door of the hut, a woman will say to her newborn, without looking up at her man, "Clap your hands, little one; your milk has arrived!"

I learned of the significance of this transaction when, in the course of instructing a young single mother of the Alto in the gentle art of breast-feeding and after many weeks of orienting her, the young girl could not contain her joy in showing me the "gift" she had just received that afternoon from a "special" person. My face fell when I saw it to be not one, but several, large cans of Nestlé's Nestogeno. The girl's shy and embarrassed excitement and palpable pleasure alerted me to smile with her, rather than spoil what was clearly a special moment. The child's father had appeared. The pretty cans of milk, displayed on the table in the front room, symbolized her newborn son's birth certificate and her own proof that she, too, was "claimed." Unfortunately, a mother who breast-feeds her infant is thought of either as an abandoned woman or as a woman whose husband does not

"provide" for her or for his offspring. It can also mean that the husband is no longer sexually "jealous" (i.e., desirous) of his woman's body, letting her breasts be "ruined" by a sucking infant. In this context, breast-feeding is a kind of social stigma.

But one can probe even further into some of the unconscious sources of Alto's women's rejection of breast-feeding. It is supported by women's mistrust of their own bodies and their abilities to nurture an infant. Most Alto women believed that they could not produce sufficient milk, that they were too poor, too weak, or too "wasted" to do so. One woman tugged at her breast, saying, "It's easy *for you* to say we ought to be feeding our own babies, but look here, our infants can suck and suck, and all they will ever get is blood or pus."

"Why is that?"

"Because we are totally weak, used up, finished, no good. We have nothing to give to our own children, not even our own milk."

Another woman broke in to say, "If we were to nurse our babies directly, we would all die of tuberculosis. Weak people can't give much milk."

When I suggested that even poorly nourished women could generally produce an abundant supply of breast milk, an older woman pulled me up short by saying, "Don't some of us raise goats, and don't we know that some goats give milk and others are dry? It is the same with us."

Nevertheless, the more that I observed infant feeding practices in Alto homes, the more I learned about the social production of scarcity. Because women were anxious about the survivability of their often small, puny infants, they hastened to offer them a heavy, thickened "pap" to fill their tiny bellies and (as we might say) to "stick to their ribs." In contrast to thickened powdered milk or even to fresh goat's milk, human milk appeared blue, thin, watery. It could not possibly be "good," Alto women reasoned. It was not rich enough or "strong" enough. It was thin and watery because they themselves were thin and had watery blood. I could not convince the women that all breast milk looked much the same as their own. In offering infants *mingau* from the very start and offering the breast only as an afterthought, Alto women could not build up an adequate supply of milk, which required active stimulation and fairly continuous sucking in the early days and weeks of breast-feeding. Within days of relative disuse, Alto women did notice a gradual decline in their breast milk and could say with reason, "I have very little milk."

What has been lost on the Alto do Cruzeiro through the commercialization of infant feeding is the whole "culture" of breast-feeding: the seemingly (but not really) "intuitive" knowledge of how breast-feeding is done, what breast milk "should" look like, how a mother knows when a small baby

has had enough, and so forth. My point is that breast-feeding is no more "natural" or any less "cultural" than cooking. Breast-feeding is a form of body praxis. Like swimming, dancing, or making love, breast-feeding must be learned, and the knowledge of "how" to do it comfortably and well (though with many cultural variations) can be lost.

Finally, many women accused their babies of "refusing" the breast when it was offered. And they could hardly blame the infant, they said, for their milk was simply "no good." I was told by dozens of women that their breast milk was "bad." They described it as salty, watery, bitter, sour, infected, dirty, and diseased. In all, the women perceived their milk as "unfit" for the infant and as little more than a vehicle of contamination, no better than the polluted waters of the Capibaribe River. Not only a woman's weakness and her sickness but even her sins could be transmitted through nursing. When a newborn died suddenly on the Alto, mothers sometimes took comfort in the belief that an unbaptized baby, if it had never been breast-fed, could go directly to heaven free of the stain of original sin passed into the infant through the mother's breast.

In all, we can see the representational uses of the body as a medium of exchange between personal and social metaphors, symbols, and meanings. When their milk supply begins to falter, Alto women are quick to see in this an indication of their own weakness. Similarly, when they refer to their own milk as scanty, curdled, bitter, or sour, breast milk is a powerful metaphor speaking to the scarcity and bitterness of their lives as women. The rejection of the "goodness" of their breast milk derives from a political economy of the emotions expressed in the somatization of scarcity and deprivation. Through the medium of the body, the contradictions of the social order are reproduced in the disquieting image of needy, hungry, dependent women who must withhold their milk from their babies to keep from being devoured by them first: "If we would feed them, they would make us sick and skinny and old." The perceived inability to breast-feed reinforces these young women's already deeply eroded sense of inner goodness, self-worth, and self-sufficiency, which is embodied in the autonomous act of nursing one's newborn. What has been taken from these women is their belief in their ability to give: "We have *nothing* to give our children." And so the cycle of economic dependency is complete. From autonomous *roçado* to commercial *supermercado*, and from autonomous breast to canned Nestogeno, the ruse is total. And its consequences are nothing less than deadly.

Social Class and Reproduction

I am suggesting that poverty interacts in many different ways to produce child mortality and to shape reproductive thinking and practice. But which is

cause? Which is effect? Do so many babies die because there are too many of them to begin with, or are there so many babies because so many of them die? Do women have more babies because they are poor? Or are they poor because they have so many babies?

In 1988 I took my study of reproductive histories into the most comfortable and bourgeois neighborhood of Bom Jesus, a half dozen suburban streets of modern, ranch-style homes with tiled patios, fringed hammocks, large swimming pools, and pet dogs. My sample of twenty-three middle- and upper-class women was, like the Alto sample, an opportunistic one. I began with a trusted friend, the wife of a middle-aged bank officer, and she introduced me to her circle of friends at a local and exclusive private social club. All members of the social club had been screened for their bourgeois credentials before being admitted. These women in turn introduced me to friends of their own.

The profile of the "average" woman in this small "control" group stands in obvious and marked contrast to that of the Alto women. The sample was drawn, after all, from the class of local *patroas*. This average affluent woman of Bom Jesus was born in the *município* of Bom Jesus on her father's *fazenda* or plantation estate, where she was reared as a favored *menina de engenho*, a true *filha da terra*, a child of the sugar mill, a daughter of the land. She was educated by German nuns at the Colégio of Santa Lúcia in Bom Jesus and then later sent off to Recife to study literature and language, history, the arts, or pedagogy. But she did not leave normal school or the university with a diploma, and she never had time to put her skills to practical use, for she returned home to marry young. Most had their first child by the age of twenty-one or twenty-two. But first she was a local debutante and had her formal "coming out" party at the age of fifteen. She dated carefully, choosing her suitors from among the "better" and "older" families of the *município*. She married "close to home," often a cousin or distant relation. Although her husband may have come from the landed "aristocracy," he is in all likelihood not an "agriculturalist" himself but a professional or a man of commerce and banking. She was not a virgin at marriage, but she was married in white in both religious and civil ceremonies and with much festivity. After marriage she settled into the comfortable, matronly role of *dona da casa*, managing a large household of family members, live-in domestics, and hired help.

The twenty-three middle- and upper-class women, ranging in age from twenty-four to forty-eight (median age, thirty-eight), reported a total of 76 pregnancies among them, or 3.3 per woman. (Compare Table 7.7 with Table 7.2.) Among them they experienced only *one* infant death (of a preterm, premature infant), one stillbirth, and seven miscarriages and abortions.

Table 7.7. Reproductive histories of middle-class women, Bom Jesus da Mata, 1988 (control group).

	Total Number	*Number per Woman*	
Pregnancies	76	3.30	
Living children	67	2.90	
Miscarriages/abortions	7	0.30	0.34
Stillbirths	1	.04	
Infant deaths (birth – 1 year)	1	.04	.04
Child deaths (13 months – 5 years)	0	0.00	

Source: Anthropological interviews of women of Bom Jesus da Mata.
Note: $N = 23$. These women ranged in age from 24 to 48, with a median age of 38.

They had 67 living children, or 2.9 per woman. Clearly, mortality and fertility patterns are thoroughly modernized in this middle-class population, for whom child death is as shocking and aberrant as for affluent women anywhere else in the world. These women do not speak of "angels" populating the heavens, nor do they take comfort in homely Catholic pieties about the afterlife and "God's will" with respect to infant and child death.

Although in this sample of women there were no infants deaths (apart from the one premature infant who died on the day of birth) to grieve, over the years I have encountered a few other cases of child death in Bom Jesus among the middle classes. Terezinha Cavalcanti, the thirty-four-year-old daughter of one of the oldest landed families in the *município*, lost her fourth child in 1981, a baby son at the age of ten months. I visited her at home a year later, and Terezinha was still in mourning for the lost child: "He was the strongest and the healthiest one of all," she told me. "As an infant he never had to see a doctor. The sickness struck without warning. He took sick overnight. He died in my arms on the way to the hospital in Recife." The child was never diagnosed, but the cause of death was a high fever with convulsions, "probably encephalitis," she said.

The family decided against an autopsy and took the baby home for an elaborate wake. His grave is well marked by a large marble headstone flanked on either side by a stone guardian angel and a replica of the family watchdog. I had occasion to visit the grave with Terezinha, who always carried photos of the baby on her person; she would take them out and show them to anyone willing to hear her short, painful story. What tormented

Terezinha was the knowledge that she could never "replace" the lost child, for at the time of his delivery she had had a tubal ligation to prevent further pregnancies. By all appearances, Terezinha was deeply depressed, and her mother often consulted me for practical advice on how she might best respond to her daughter's suffering.

Similarly, a local doctor in Bom Jesus who had lost a five-year-old son to leukemia was "inconsolable" for months following the death. The child was his firstborn son and his namesake. No one in Bom Jesus seemed able to help the man overcome his grief. Finally, the doctor went to the famous archbishop of Recife-Olinda, Dom Helder Câmara, for spiritual advice and comfort. There he asked the elderly "saint" if the death of this favorite child was "God's will" to which he must somehow learn to adjust. Dom Helder shocked the grieving man by saying, "God is not a cannibal—he doesn't wish any child to die. The death of your son was a terrible *human* tragedy, not the workings of a willful or capricious Deity. But let us pray for the strength to pass through this terrible hour." Both men knelt together and wept, Dom Helder as much as the doctor, for his grief recalled to the elderly prelate the death of six of his own thirteen siblings during an epidemic of "croup" when Dom Helder was still himself a child.

Middle-class infants and small babies are not completely protected from the scourge of life-threatening diarrheas, fevers, respiratory infections, and other pediatric diseases. Although there was only one death, two other women interviewed came very close to losing a child, one to croup and another to diarrhea/dehydration. Both infants were rescued, however, by prompt and appropriate medical attention in Bom Jesus. And several of the middle-class mothers boasted that none of their children was ever put on a course of antibiotics, this in contrast to the babies of the Alto, who were fed a steady diet of medications, including many different varieties of strong antibiotics.

Among the affluent women of Bom Jesus, reproduction was described as a controlled and rationalized, rather than a chaotic and fearful, process. All births took place in urban hospitals and maternity centers, and a large number of these were cesarean sections. While interviewing a middle-class woman on her patio, I noticed her six-year-old daughter and friends playing "hospital." We interrupted the interview so that I could watch the play-birth just as the "nurse" called for the "doctor" to deliver the baby. The mother was anesthetized, her belly was cut open, and the "birth" was pronounced a success by the little girls, although the infant was immediately put on intravenous feedings! Because surgical births have become so normalized among the middle and upper classes of Northeast Brazil (see Janowitz 1982)—and newspapers in Recife reported C-section rates approaching 70

percent for private maternity patients—poor women frequently complained of medical "maltreatment" at the local maternity ward: they were forced to give birth "naturally," although sedated.

If Women Could Choose: Fertility, Mortality, and Ideal Family Size

> Had those women on the Alto [do Cruzeiro] refrained from the momentary, fleeting pleasures of irresponsible orgasms, there would be no dead babies to bury. Furthermore, by solidifying commitment, one man to one woman, until death do they part, by practicing abstinence and responsible sex, a way out of poverty and oppression might more easily be found.
>
> Unpublished letter in response to Nancy Scheper-Hughes (1989)

During a lull in the conversation at a diocesan dinner for the bishop of Oakland attended by invited Catholic faculty of the University of California at Berkeley in the fall of 1989, I was asked about my research interests by the Dominican philosopher seated at my right. I summarized some of the foregoing figures on fertility and mortality. The Dominican philosopher's response was learned but coolly indifferent. He said, "Interesting, but throughout history poor women have probably always lost more children than they were able to raise. No doubt they are more accustomed to death than we are." Indeed, they are, I replied, but at great cost to their bodies and their spirits. Our conversation was cut short, however, for just then the bishop of Oakland got up to address the gathering, managing skillfully to sidestep the critical questions from the floor on the church's continuing condemnation of artificial birth control, abortion, and education for "safe sex" in response to the current AIDS crisis. It was not a terribly friendly audience.

We know that educated Catholics in North America, long exposed to a religiously pluralistic environment, exercise a good deal of independence from official church teachings on sexual and reproductive ethics. But to what extent do these same teachings influence reproductive thinking and practice in Bom Jesus, where 85 percent of the population describes itself as Catholic? Do the words of Pope John Paul II fall on receptive and fertile ground here, and do they contribute to the seemingly self-defeating tendencies of poor Alto women who plunge themselves into a lockstep dance of repeated pregnancy and repeated child death?

In short, why *do* these poor Brazilian women have so many babies? Are they ignorant of birth control methods, or are they spiritually battered by

Table 7.8. "If women could choose": ideal family size.

	Total Pregnancies	*Living Children*	*Ideal Family Size*
Middle-class women	3.3	2.9	4.5
Alto women	9.5	4.2	3.0

Source: Anthropological interviews of Alto and Bom Jesus women.

the sterile and uncompassionate teachings of the Catholic church with respect to sexuality? Are they also passive objects of the reproductive whims of selfish husbands and lovers? Alternatively, do these women simply value fertility, pregnancy, childbearing, and childrearing to the extent that they "overproduce" for the "love" of it? Or, like Alice in *Through the Looking Glass* racing just to stay in place, do these women give birth again and again only to hold on to the precious few who manage to escape death?

A frequent response worldwide to infant death is pregnancy, an attempt to quickly replace what was lost. The evidence of high infant mortality "driving" up rates of fertility is generally accepted today by most demographers and epidemiologists (see Imhof 1985; Ware 1977; Choudhury, Khan, & Chen 1976). The reproductive histories of Alto women represent, in plain terms, gross reproductive waste, draining the physical, economic, social, and psychological resources of women who bear again and again such vulnerable offspring. But how many children do Alto women actually want? And how much are they willing to go through to achieve that goal? Do the women of the shantytown differ significantly in their reproductive goals, practices, and outcomes from middle-class women in the town of Bom Jesus da Mata? Here, comparison across class in the same small community can show the effects of scarcity on reproductive thinking and practice and help us to tease apart what is culturally shared from what is class driven.

In all, while it takes the "average" Alto woman almost 10 pregnancies to rear 4 living children, middle-class women of Bom Jesus have 3.3 pregnancies to rear 2.9 living children (see Table 7.8). In other words, the average Alto woman labors through 6 more pregnancies than the average middle-class woman to have 1 child in excess of her more affluent "sisters." The women of the Alto, then, are not prodigious reproducers; they must simply work much harder than affluent women to produce a medium-sized family.

When asked what each considered an ideal-sized family, the poor women of the Alto expressed a preference for many fewer children than the number of pregnancies they had experienced. Poor women also wanted fewer chil-

dren (3.0) than did middle-class women (4.5). Whereas the small sample of affluent women clustered closely around the mean age of thirty-eight, the Alto women represented two generations of women, an older, postreproductive group and a younger, still fertile group. So, referring back to Table 7.4, we can see that the younger generation of Alto women actually considered 2.6 an ideal family size, while for those older Alto women who agreed to answer this "irrelevant" question, 3.5 was the ideal-sized family. Here is what a few younger Alto women had to say about family size:

"A pair; that's plenty for a poor family" (aged thirty-two).

"Two is good; if I could, I would have only one boy and one girl" (aged nineteen).

"Three or four—that's enough!" (aged twenty-four).

"Two are more than enough these days" (aged twenty-five).

"More than three is an agony. I've had seven . . . too many, too many" (aged thirty-three).

"Two of each sex. When you have too many children, you are in the middle of a great confusion. You don't know where to turn. If one wants something and you give it to him you end up taking something away from another" (aged twenty-nine).

"Whatever God decides, that is good. But for myself, what *I* would have liked was two of each sex, and all of them living" (aged thirty-one, lost seven of ten children).

And here is what a few of the older women of the Alto (forty and older) had to say:

"Three would be good. If there had been only three in my family, we could have lived better. But God didn't want this—He gave me all of these" (aged forty-four, mother of nine children, six living).

"Two are plenty for us. Everything for the poor is a struggle. We want to put them into school, and it is a sacrifice. We want to feed them and dress them, and we can't" (aged seventy).

"Ten, at least ten children. I would have preferred to have raised all of mine, to have lost none of them. I gave birth nine times, and I raised five of them, in addition to two others that belonged to my husband from another woman. They were good children; I loved all of them" (aged fifty-six).

"Six. Three boys for their father in the *roçado* and three girls for me at home" (aged fifty-two).

"Four or less—more than that and they start to die on you" (aged forty-seven).

Those older women who "refused" the question replied indirectly:

"Ideal family size? That's hard to say. I myself had so many that I am tired, worth nothing anymore. My desire was always to conform to God's will because all that God does is good. I love to hear the words of the holy Mass wishing goodwill to all. It is a small sacrifice, then, to conform to His wishes" (aged seventy-six).

"Whatever God sends us is good" (aged seventy-four).

"It's not for us to say. But sometimes I think it would have been better if God had decided to take them all, though He was right to leave me with just these two. They served me well" (aged sixty-six).

In general, Alto women were more conservative than middle-class women on the subject of birth control. "Who doesn't want children shouldn't marry," older Alto women often said. But poor women were also pragmatists who understood that unlimited fertility was not an unlimited good: "I hope to God that I won't have any more children," a forty-two-year-old Alto woman offered. "But I won't do anything to prevent it either. If God is willing, then I will have to be willing, too."

Nonetheless, *most* poor women younger than forty had had some experience with birth control, usually the pill.[8] Many had used the pill for brief periods of birth spacing but rarely for more than six to eight months at a time. The pill was described as an effective but "dangerous" form of birth control. It was uniformly disliked by Alto women, an attitude widely shared among all women of Bom Jesus across class. The pill was said to cause nausea, headaches, swelling, extreme nervousness . . . and cancer. While readily available in most pharmacies of Bom Jesus and sold without prescription for a modest fee that would *not* be entirely prohibitive to the poor, the pill was considered an "unnatural" form of birth control that adversely affected, as one Alto woman put it, "the whole organization of the nervous system." Contraceptive creams, jellies, and pastes were known and sometimes used by the younger women of the Alto. Diaphragms were unknown and unavailable in local drugstores, while condoms were generally used only by local men when visiting prostitutes in the *zona* of Bom Jesus.

The older generation of Alto women had had recourse to a vast array of herbal teas, washes, baths, and infusions said to affect fertility in a variety of ways but most commonly to "bring on" a late period. These traditional remedies were never referred to as having "abortive" properties but rather as "regulating the normal menstrual cycle. Sister Christiana, another Franciscan nun who ministers to the sick and especially to women and small children of the poor *bairros* of the parish, is well versed in the medicinal and fertility-regulating qualities of various indigenous plants, a knowledge she readily shares with the young women of the Alto. Sister Christiana describes the *mata* (the wilds) as "God's own pharmacy," and she teaches poor

women at religious base community meetings that whatever can be effected through "natural" herbs and plants is "good," an expression of God the Creator's intent.

Little Irene and I went to each of the half dozen medicinal herbal stalls in the *feira*, where we collected samples of menstrual-inducing herbs, leaves, roots, and bark. There we encountered Dr. Raiz (Dr. Roots), who, reluctantly at first, told us about the properties of various abortifacient plants, among them *cabacinha* (also called *cabacinha-riscada* and *aboboraovos*), *senna* (any one of several species of cassia); *quinaquina* (*cinchona offinalis*, also known as *guamixinga* and *murta-do-mato*). Dr. Raiz believed that his vast knowledge of healing herbs, roots, and barks (several hundred of them) was a gift from God and that his was an *obra da natureza*, a "work of nature." Therefore, he said, he would not "cooperate" with those who asked him to use his gift for ill. He would only "treat" women for menstrual periods up to fifteen days late. After that date the women would have to look "elsewhere" for help, perhaps (he said) with a local midwife or a curing woman willing to use her knowledge in that way. Or the women could go to Feliciano's pharmacy for an abortion injection or an abortion "cocktail."

Later, I visited several local healing women (*rezadeiras*, *parteiras*, and *curandeiras*) of the Alto, and we walked the *mata* looking for samples of green herbs, berries, nuts, roots, and leaves used to stimulate delayed menstrual periods. Four more abortifacients were added to the list in this way: *aroeira* (*Schinus molle*), which is used either as a tea or a douche; *caju roxo* (a variety of the common cashew); *arruda* (common rue used as a douche); and *cravo do reino* (a species of clove). To what extent these natural remedies were efficacious, I could not say. Nonetheless, the knowledge and practice of menstrual induction were fairly widespread among the women of the Alto and known even to some of the religious sisters of Bom Jesus, who, like Christiana, participated in the popular church's new alternative medicine ministry to the sick-poor. In so doing the women (including the nuns) acted with considerable independence from some central tenets of Catholic orthodoxy regarding the nature of the embryo and the beginnings of human life. They did so by taking advantage of the considerable ambiguity surrounding menstruation. A late period might be seen as the first sign of pregnancy *or* as the symptom of a sluggish system requiring an herbal purgative.

It seems, then, that menstrual-inducing pharmaceuticals, like Depo-Provera, might be compatible with this orientation. And, indeed, several younger women of the Alto said that they had used the "injection method" of birth control. Despite the controversy surrounding Depo-Provera in the United States because of its links to uterine cancer (see Petchesky 1984:8),

the drug was not listed as a "controlled" or "regulated" substance on the Brazilian government's published inventory. The drug was sold at local pharmacies in three different injectable dosages, and I purchased a small sample injection without prescription at Rute's pharmacy. Rute, who was strongly opposed to abortion, did not associate the drug with its abortifacient effects. She believed that the drug was recommended to "promote fertility" by normalizing irregular menstrual cycles. Indeed, the accompanying directions indicated its use for the control of endometriosis and the treatment of amenorrhea, the latter an indirect reference to the drug's abortive properties.

There was another way that Alto women manipulated ambiguity with respect to induced abortion. Most women knew that it was dangerous to take any medications in the early stages of pregnancy. Because, however, so many poor women *so anda doente*, were constantly sick and "eating" so many different and often "contradictory" drugs, abortion sometimes resulted as the "unintended" consequence, a side effect, of their tendency to overmedicate. Such was the case with Little Irene, who, as you recall, was quite ill when I found her on that morning in 1982 and asked her to be my field assistant. Irene had many different somatic complaints; a suspected pregnancy and an induced "miscarriage" were only two of her troubles. As we plunged into the first few days of interviewing, Irene frequently alluded in an offhand and lighthearted way to the unwanted pregnancy she had just been relieved of. During an interview in the creche, Marlene asked Irene about her pregnancy. Irene replied, "I'm free! It was because of all the drugs I was taking. I'm like a walking drugstore! I'm sick with so many different problems I finally just took everything at once. I made myself a drug cocktail and *pronto!* The next morning when I went to relieve myself—plop! There it was, a little sack of white stuff this long. [She indicated that it was half as long as her index finger.] For sure it was all those medicines I took."

Marlene exclaimed, "For shame, Irene. What a *big* sin!"

Irene replied without skipping a beat, "There was no sin at all! I'm no more responsible for this abortion than a woman who lost hers because of a great *susto!* I was sick, doubled over with pain. I thought I was going to die. And now I'm totally cured! How could that be wrong? I have three other children still at home to look after."

Nonetheless, virtually all Alto women (Irene included) vigorously condemned medically induced abortion, whether by surgical procedures in the hospital or by the manipulations of local midwives at home. Fully intentional abortions like these constituted a grave sin, an offense against God and the Virgin. "Once it's *really* in there," Alto women cautioned, "it's too late." Because medical abortions in Bom Jesus required several weeks of gestation,

the ambiguous period immediately following a "late period" had passed by the time of such an abortion. Nothing is less ambiguous than a clinical abortion, and those half dozen Alto women in my sample who admitted to having an abortion considered themselves guilty of a mortal sin. Abortion, they maintained, was morally wrong (as well as illegal in Brazil), even if dire necessity had led them to it. The soul of an unbaptized, aborted fetus was an anomaly, the soul of a child unborn, and older praying women warned that such restless souls hovered near crossroads and sometimes got into mischief: "They hold it against their mothers—they never forget who brought them to their end." Consequently, induced abortions were often held responsible for all quality of misfortune in the world: drought, floods, evil landlords, the deaths of older children, and disease epidemics. On this point the women of the Alto stood strongly by orthodox church teachings, and they often said quite straightforwardly, "The pope forbids it."

The small, mainly evangelical Protestant churches of Bom Jesus also forbade abortion among their members, although these churches had not yet gained a foothold on the Alto do Cruzeiro. There, Catholicism competed only with Xangô and Umbanda, the Afro-Brazilian possession religions, and these, too, both strongly condemned abortion.[9] Nita the Marvelous explained that Exu, the trickster (devil and messenger) of Umbanda, was legion and was always accompanied by his "women" and their host of pagan child spirits. These *almas inválidas* (disabled souls) belonged to the multitude of aborted and unbaptized fetuses. "Beware these evil spirits," warned Nita, "for they are the most dangerous of all." The baptized spirits that did Exu's bidding were civilized, endowed with reason and with a knowledge of good and evil, and they could discriminate between the two. But the mischief of the crippled pagan souls of the unbaptized and unborn knew no bounds. They were primitive spirits, poorly evolved, and totally unsocialized, said Nita, and they were to blame for much evil in the world. "And yet," she sighed, "they are not the guilty ones. They are not responsible for what they do. They have no conscience. Who are to blame are their mothers, those base and low-life women who destroy their progeny in the womb. And in the end, *they* are the ones who will suffer because they will go into the next world *devendo o espíritu a Deus* [owing a soul to God]."

In contrast to the moral sensibilities regarding abortion, sterilization emerged as the preferred form of birth control among women of all social classes in Bom Jesus even though sterilization was viewed, equal to abortion, as a serious mortal sin by the Catholic church. For middle-class women who were able to afford it, sterilization was also the most commonly practiced form of family planning. Similarly, while local doctors expressed considerable moral conflict over medicalized abortions, they expressed no qualms

about sterilization once a woman had demonstrated her "good gender citizenship" by producing an "adequate" number of children. Often I was asked to serve as a *despachante* in arranging, through various political and medical "connections," for the free sterilization of an Alto woman who could not afford to pay for her own. Political leaders in Bom Jesus complained that expensive sterilizations (at about one hundred fifty dollars each) were among the most popular political "favors" distributed to poor women voters. Nevertheless, Seu Félix denied that local women ever received the operations at municipal expense . . . except during municipal elections when he was forced to keep pace with his political opponents.

The women of Bom Jesus, both rich and poor, did not see any contradiction in their moral condemnation of medical abortion and their widespread acceptance of sterilization. The latter, they said, did not involve "killing" a fetus. And tubal ligations were second only to delivery as the most common medical practice for women inpatients as the local hospital. Overall, however, the morality that guided reproductive thinking and practice for both rich and poor women in Bom Jesus was contained within a very Catholic discourse on the "natural" (as though with oblique reference to a kind of Thomistic natural law). Women, they agreed, had a duty to "cooperate" with God and with nature. This was equivalent to a "duty to procreate" (see Noonan 1966:193), although not indefinitely. At a certain point a womanly pragmatics took over. Once a woman had had four or five or twelve children, she might decide to put an end to her reproductive years. A twenty-seven-year-old Alto woman, an active member of the women's altar society of the local church and pregnant herself for the second time, could say; "I want only these two children. I hope to have my tubes tied while I am in the hospital for the birth. My husband has agreed. The only problem is with the hospital. The doctors don't like to do these operations for those of us who can only pay the minimum allowed through the rural workers medical fund. They make the poor suffer through seven or eight pregnancies before they finally take pity on us."

The young woman was correct. Unless one was very well connected politically, it was almost impossible to get a tubal ligation after only two pregnancies. Consequently, only 13 percent of the seventy-two Alto women were able to arrange for their sterilization, although many more had tried to do so. By contrast, fourteen of the twenty-three middle-class women interviewed had already had tubal ligations, which they viewed as the expected and normative practice of family planning among their peers. Among the middle and upper classes of Bom Jesus, once a woman and her husband had agreed that they had had enough children, a tubal ligation was scheduled, along with a "uterus-lifting" and "vaginal-tightening" operation that had

become fashionable among middle-class women wanting to maintain their husbands' sexual interest. "No man likes a flabby old woman," I was told more than once with a wink. Male sterilization, however, had made no inroads into Bom Jesus at all. Only one local man, a lawyer, was known to have had a vasectomy, and he was ridiculed as a man who had been too willing to sacrifice himself to a form of symbolic castration.

While middle-class women of Bom Jesus were strong believers in "family planning" for themselves, they were even stronger supporters of "population control" for the poorer classes of the town. And while Alto women emphasized pregnancy and childbearing as a womanly "duty," the middle-class and wealthy women of Bom Jesus tended to view procreation as a "right" to be exercised only when there were sufficient resources to assure each child a decent upbringing. In response to my question "How many children are good for a poor family?" one middle-class woman snapped, "*None*, not one!" Claudinette was more gentle but firm: "If it is a sin to say this, forgive me, but I think that for a poor woman to raise a child only so that it will suffer and go hungry, I think it would be better if that child had never been born."

Many middle-class women interviewed shared the belief that the negative teachings of the Catholic church had a greater effect on the poor, who were more "vulnerable" and at the same time more "devout" and "obedient." Dona Elizabete pulled from her purse a small pamphlet that had been distributed at Mass on the previous Sunday. It had formed the basis of the weekly sermon, which had treated the subject of "chastity," sexual purity, within and outside of marriage. The pamphlet descried a decline in the spirit of self-mortification in the modern world and the proliferation of sinful practices: promiscuity, fornication, adultery, homosexuality, bestiality, masturbation, and so on. It condemned even within marriage any sexual practices that deviated from "natural" law, such as anal intercourse, the use of sexual stimulants (whether dildos or erotic videotapes), and artificial birth control. Elizabete was incensed at a message that equated bestiality and masturbation, on the one hand, and "pornography" and birth control, on the other. She believed that the church's misguided efforts would be ridiculed by sophisticated Catholics and uncritically accepted by the illiterate popular classes. Either way, she said, it was a disservice to the church in Brazil. Herself the mother of four grown children, Elizabete had her own tubes tied when she was only thirty-four years old. "I know that the pope is against this," she said, "but this is a sin that I will have to carry with me." Had she confessed it to the local priest?" "No," she replied. "You only confess sins that you repent." Did she still receive Holy Communion? "Yes," she answered. "This matter concerns me and God alone."

In all, the independent reproductive practices of the middle classes of Bom Jesus are not too dissimilar from those of North American Catholics. Those of the poor are another matter. If official church teachings affect their reproductive patterns, contributing to the high fertility/high mortality cycle that I have described for the women of the Alto do Cruzeiro, the church has the devil to pay in terms of accountability. But Catholic teachings are deflected and creatively refashioned on the Alto within the context of a lively popular culture of spirituality and an alternative, folk bioethics concerning the definitions, meanings, and value of early human life (to which I now turn).